

CONTENTS

Preface	i
<u>PART 1: Assessment of Services Delivered by CSOs</u>	
Executive Summary	2
Introduction	8
Literature Review	15
Methods and Materials	30
Results	34
Discussion & Conclusion	83
Recommendations	88
References	
Appendices	

EXECUTIVE SUMMARY

The World Health Organization (WHO) declared the Coronavirus disease 2019 (COVID-19) a pandemic on March 11, 2020. However, Jamaica's first medically confirmed case was on March 10, 2020. Globally, since March 2020, there have been 632,533,408 confirmed cases of COVID-19 and some 6,593,715 deaths as of November 9, 2022. Jamaica has had 151,931 confirmed COVID-19 cases and 3,320 deaths. This pandemic has claimed the lives of many peoples across the globe. The mortality of people included children, the elderly, those with comorbidities, and people in vulnerable populations.

In attempting to protect the lives of humans, governments across the globe introduced many measures to curtail the spread of COVID-19. Some of those precautionary measures included social distancing, wearing a mask, hand sanitization, lockdowns, vaccination, and working from home. The lockdowns substantially curtailed economic activities, and many people lost their economic livelihood during the process. The matter was even more intense among the vulnerable populations—the unemployable, sick, shut-ins, children, and so forth.

The economic realities during the COVID-19 pandemic have created a burden on vulnerable populations. These have intensified the challenges of those vulnerable populations, particularly those in developing nations. Many vulnerable people were struggling to meet their daily existence, and this was even more devastating during the pandemic.

COVID-19 increases the burden among those with comorbidities including those with HIV/AIDS. The Global Fund realized the challenges caused by COVID-19 to the well-being and existence of those living with HIV/AIDs as well as other vulnerable populations and disbursed funds to nations across the globe for alleviating the challenges experienced by vulnerable populations. The Ministry of Health and Wellness (MOHW) was the agency that received the

funds from the Global Fund and then disbursed this to civil society organisations (CSOs) in Jamaica. The recipients were the Jamaica AIDS Support for Life, Jamaicans for Justice, ASHE Company, Eve for Life, Jamaican Network for Seropositives, Jamaica Community of Positive Women, Transwave Equality for All Foundation, and Children First Agency. This raises the issue of how the funds were spent and whether the vulnerable groups were well-served during the pandemic. Therefore, this study aims to evaluate the following research questions:

- 1) How were the service delivery and well-being affected by the advent of the COVID-19 pandemic?
- 2) What was the degree of interference of COVID-19 on the CSOs' service deliveries in Jamaica?
- 3) How did the grant from the Global Fund assist CSOs in overcoming the disruption to their critical services?
- 4) Were the measures introduced during the COVID-19 pandemic able to effectively meet the needs of clients?
- 5) How was the preparedness of the CSOs for a pandemic prior to the COVID-19 pandemic?
- 6) How are policymakers and stakeholders to structure themselves to deal with future pandemics?

For us to get a comprehensive understanding of any phenomenon one must examine it from different perspectives, and this must be done by way of more than one method/approach. A hybrid research approach offers more truths. The hybrid approach is done by twinning methods, which allows for triangulating truths and provides in-depth knowledge of the studied phenomenon. With the hybrid approach providing more in-depth knowledge of a phenomenon, this research employed both an objective (i.e., quantitative) and a subjective (i.e., qualitative) approach. The hybrid approach is to assess the services delivered by selected civil society organisations (CSOs) that

received funding from the Ministry of Health and Wellness which is made possible by the Global Fund to aid vulnerable groups during the COVID-19 pandemic.

Summary Findings

- Of the 80 sampled clients of CBOs, the response rate to the question ‘Did you receive support from a CSO during the pandemic?’ was 97.5 per cent (n=78). Of the 78 clients, 91.03 per cent indicated that they have received support during the COVID-19 pandemic.
- Fifty-six per cent (56%) of the sampled clients (n=44) rated the services received from the CSOs **before** the COVID-19 pandemic as excellent compared to 15.4% (12) who said very good. Only 2.6% of the sampled clients indicated that the service received was fair (n=2), and 3.8% (n=3) stated very poorly. This means that 71.8% stated that the service delivery was at least very good compared to 6.4% who indicated it was at most fair before the pandemic.
- In addition, most (83.53%, n=66; Figure 4) of the sampled clients indicated that they received at least good service from the CSOs **during** the COVID-19 pandemic, with 38% stating that this was excellent. The sampled clients indicated that the quality of the services received from the CSOs during COVID-19 had fallen (i.e., 32.6%; Table 6), and that poor service has increased by 65.8%.
- Of the sampled clients (n=80), 96.25% (n=77) responded to the question ‘Was there a break in services you received from the CSOs during the COVID-19 pandemic?’ Fifty-six per cent (56%) of the sampled clients indicated that there was no break in service received from their CSOs during the COVID-19 pandemic. On the other hand, 40% of sampled clients indicated a break in service delivered by their CSOs during the COVID-19 pandemic.

- There is a high level of satisfaction with the degree of services received from the CSOs both before (86.7%) and during (89.3%) the pandemic, with marginally more clients being pleased with the service delivery during the pandemic compared to before the pandemic. The largest plurality of clients (17.1%) rated their level of satisfaction as 100% followed by 12.9% who expressed 90% satisfaction and 12.9 % who registered 80 per cent satisfaction. The pictorial display of the level of satisfaction of the clients speaks to a high degree of gratitude and fulfilment of needs by the CSOs during the pandemic, and pleasure derived from the work done by the CSOs during the pandemic. Overall, the average level of satisfaction with the services provided by the CSOs was 80% (range = 100%), with 68.6% rating their level of satisfaction to be at least 70%. Furthermore, 21.4 % of the clients gave their level of satisfaction to be at most 40%.
- The findings revealed the officers of the CSOs' responses to the question: "During the pandemic, was there a break in your services?" Some (33.3%) admitted that there was a break in their service delivery during the pandemic. Others indicated that their services were continuous during the COVID-19 pandemic. In addition, the officers (67%) of the CSOs indicated that COVID-19 had significantly hampered the **extent** of their service delivery, and only 33% stated that was not the case.
- The officers of the CSOs were asked, "What aspect of your programme was most affected? They indicated three of the four options (i.e., prevention, treatment, and enabling environment). None of the officers indicated that the rights of the clients were affected during the COVID-19 pandemic. Sixty-seven per cent (67%) of the officers of the CSOs indicated that the grant funding was adequate to meet their programmed needs for the pandemic. This means that 33% of the CSOs were faced with the challenge of inadequate

funding to address the needs of their clients during the pandemic. In fact, the CSOs indicated that the COVID-19 response funds arrived towards the latter part of the pandemic. Hence, other sources of funds were used to front load the responses to the disruption of service delivery.

- The findings revealed that zero per cent of the CSOs fulfilled 100% of their service delivery during the COVID-19 pandemic. However, the CSOs indicated that between 44% and 88% of their targeted service deliverables were accomplished during the pandemic: 35% deliverables at levels of 45-65 %, and 85% deliverables at the level of 87% during the pandemic. There is a convergence between the services delivered by the CSOs and services received by the clients of the CSOs. Both parties agreed with the service deliverables and receivables during the COVID-19 pandemic.
- COVID-19 has changed the geopolitical and social landscape of our contemporary society. The pandemic has intensified the economic burden of the vulnerable population, and this has increased among those with comorbidities. The Global Fund COVID-19 response was timely, and it provided needed alleviation for the vulnerable population. The vulnerable population in Jamaica experienced many socioeconomic challenges during the COVID-19 pandemic, and civil society organizations (CSOs) were able to navigate many of those issues for this population.
- The at-risk population in Jamaica benefited from the contribution, efforts, and work of the CSOs during the pandemic. This cohort of Jamaicans indicated a high level of satisfaction with the services received from the CSOs. Although the CSOs were hampered by the COVID-19 governmental protocols and internal challenges such as infected workers, they were able to provide needed services to the at-risk population.

- The study revealed that CSOs were significantly unprepared for the disruption to their operations caused by the COVID-19 pandemic, and that there is need for policymakers and stakeholders to be better prepared to handle the challenges from future hazards at the scale of a pandemic or less.

PART 1: Assessment of Services Delivered by CSOs

Introduction

There is an increased dependency on community-based organizations (CBOs) or civil society organizations (CSOs) in times of crisis. As the world grappled with global crises, CSOs are needed to provide for the unmet needs of vulnerable communities and people. These services provide intimate, person-centred care to reduce health and human-social disparities and improve people's overall quality of life worldwide. There is a greater dependence on these services as parental guidance and community cohesion are increasingly being eroded. These services target different areas of society and focus on the most dominant issues they face. CSOs promote, provide or otherwise address behavioural challenges, healthcare and nutrition, housing and mental health issues, et cetera.

The coronavirus disease (COVID-19), which is caused by the SARS-CoV-2 virus, has transformed our social world and dominated the news in 2020 onwards. Globally, COVID-19 has infected some 620 million people and there have been approximately 6.5 million deaths caused by the virus as of October 14, 2022. In Jamaica, some 151,931 cases of COVID-19 have been confirmed, with some 3,320 deaths in the previously stated period. The World Health Organization (WHO) declared the COVID-19 virus a pandemic on March 11, 2020. The day before the pandemic declaration, Jamaica had its first confirmed case of COVID-19.

Before 2020, Jamaica had been experiencing many challenges including crime, poverty, communicable disease, missing children, battered women, and sexual exploitation of females. The measures recommended by the WHO and the Centers for Disease Control and Prevention (CDC) to address the virus were implemented by many governments across the globe including lockdowns. The lockdowns create new challenges such as increased unemployment, lower economic

opportunities, disruption in key services, increased cases of missing people, more conflicts in the homes, and reduced outdoor entertainment, relaxation, and physical social interaction. The lockdowns were creating many other social issues, and they were also affecting many of the services provided to vulnerable groups in society (i.e., children, HIV/AIDS patients, et cetera), and these were intensified during 2020 and 2021.

Help was needed to address some of the challenges caused by the pandemic, and the Global Fund was there to assist countries to mitigate the impact of COVID-19. The COVID-19 Mechanism (C19RM) provided funds to fight HIV, TB and malaria and improve nations' health and community systems. The Jamaican Ministry of Health and Wellness benefited from the Global Fund, and the funds were disbursed to civil society organizations (CSOs) across Jamaica. The Jamaica AIDS Support for Life (JASL), Jamaicans For Justice (JFJ), Jamaican Network for Seropositives (JN+), Jamaica Community of Positive Women (JCW+), Transwave Jamaica, Equality For All Foundation, Eve For Life, ASHE Company, and Children First Agency received Global Fund COVID-19 Response. The money was spent by the CSOs to respond to the challenges caused by the pandemic. Financial prudence requires accountability, so this study assesses the resources used, challenges encountered, and a crisis management pathway that can be employed by CSOs in the future to address a similar reality of COVID-19. The next section of this study will review AIDS & HIV in Jamaica because of the importance of continued servicing of the patients to life followed by the other CSOs who received Global Fund COVID-19 Response from the Ministry of Health and Wellness.

Historical background of AIDS & HIV

Acquired immunodeficiency syndrome (**AIDS**) has significantly impacted the global human population from when it was first diagnosed case of AIDS was in June 1981 in the United

States (i.e. Washington), and the number of mortality by this virus has exponentially increased compared to the 1980s [1-4]. The Jamaican Ministry of Health and Wellness reported that the nation's first diagnosed case was in 1982 [5] and that approximately 1.7 per cent of the Jamaican population is diagnosed with **human immunodeficiency virus (HIV)/AIDs**. Professor Peter Figueroa indicated that in 2018, the number of HIV cases was estimated to be 1.5 per cent of the Jamaican population (i.e. 32 617). Jamaica like the United States has seen an exponential rise in the number of HIV/AIDS cases since the 1980s [6, 7].

Using HIV/AIDS data from the Jamaica Ministry of Health for the period 1982-2001, Olukoga [7] showed that 60.22 per cent of the reported cases were for males and 37.78 per cent were for females and the majority of cases were Jamaicans ages 20-54 years old. The figures for HIV/AIDs in Jamaica are a rationale for many interventions that have been implemented over the decades. Jamaica's Global AIDS Response Progress Report [8] indicated that in 2015, there was a targeted 50-per cent reduction in sexual transmission of HIV.

Despite the numerous social intervention programmes that have been launched and implemented by the Jamaican government, a report by the Ministry of Health indicated that "Jamaica has an estimated HIV prevalence of 1.6% among the general population. Based on both modelled estimates and the case-based surveillance data, it is estimated that 29,000 persons are currently living with HIV in Jamaica; but approximately 16% are unaware of their status" [9]. Despite the various intervention programmes, Jamaica has only seen a 3.6-per cent decline in the number of reported HIV cases for 2017 over 2016 and a 16.9 per cent rise in cases in 2016 over 2015 (**Table 1**) [10-14]. Jamaica has seen a marginal decline in the number of people living with HIV/AIDS in 2017 (**Table 1**); but there has been a smaller decline in the number of deaths by AIDS for the sample period (i.e., 2.8%, Table 2).

Table 1:

Summary of HIV cases diagnosed by Year and Gender, Jamaica, 1982-2017

Year	Male	Annual % Change*	Female	Annual % Change*	Unknown	Annual % Change*	Total	Annual % Change*
1982-96	2770		1613		37		4420	
1997	698	-74.80	447	-72.29	2	-94.59	1147	-74.05
1998	652	-6.59	460	2.91	1	-50.00	1113	-2.96
1999	831	27.45	651	41.52	2	100.00	1484	33.33
2000	853	2.65	714	9.68	0	-100.00	1567	5.59
2001	795	-6.80	791	10.78	5	∞	1591	1.53
2002	729	-8.30	681	-13.91	1	-80.00	1411	-11.31
2003	844	15.78	838	23.05	1	0.00	1683	19.28
2004	865	2.49	1023	22.08	0	-100.00	1888	12.18
2005	896	3.58	1063	3.91	0	∞	1959	3.76
2006	994	10.94	1063	0.00	1	∞	2058	5.05
2007	823	-17.20	879	-17.31	0	-100.00	1702	-17.3
2008	944	14.70	1040	18.32	2	∞	1986	16.69
2009	798	-15.47	859	-17.40	0	-100.00	1657	-16.57
2010	849	6.39	782	-8.96	0	∞	1631	-1.57
2011	823	-3.06	850	8.70	0	∞	1673	2.58
2012	835	1.46	793	-6.71	0	∞	1628	-2.69
2013	680	-18.56	586	-26.10	0	∞	1266	-22.24
2014	585	-13.97	604	3.07	0	∞	1189	-6.08
2015	543	-7.18	519	-14.07	0	∞	1062	-10.68
2016	661	21.73	580	11.75	0	∞	1241	16.85
2017	621	-6.05	576	-0.69	0	∞	1197	-3.55
2018							1600 ¹	
2019								
2020								
Total	19089		17412		52		36553	

Source: [13]

¹[10]

*Computed by Paul Andrew Bourne

Table 2:

Summary of AIDS death- Jamaica, 1982-2017

Year	Male	Annual % Change*	Female	Annual % Change*	Total	Annual % Change*
1982-96	768		428		1196	
1997	248	-67.71	145	-66.12	393	-67.14
1998	233	-6.05	142	-2.07	375	-4.58
1999	341	46.35	207	45.77	548	46.13
2000	359	5.28	257	24.15	616	12.41
2001	329	-8.36	258	0.39	587	-4.71
2002	402	22.19	285	10.47	687	17.04
2003	380	-5.47	269	-5.61	649	-5.53
2004	378	-0.53	285	5.95	663	2.16
2005	306	-19.05	203	-28.77	509	-23.23
2006	263	-14.05	168	-17.24	431	-15.32
2007	202	-23.19	117	-30.36	319	-25.99
2008	236	16.83	164	40.17	400	25.39
2009	234	-0.85	143	-12.80	377	-5.75
2010	198	-15.38	135	-5.59	333	-11.67
2011	234	18.18	158	17.04	392	17.72
2012	155	-33.76	101	-36.08	256	-34.69
2013	169	9.03	129	27.72	298	16.41
2014	121	-28.40	96	-25.58	217	-27.18
2015	136	12.40	118	22.92	254	17.05
2016	185	36.03	133	12.71	318	25.20
2017	170	-8.11	139	4.51	309	-2.83
2018						
2019						
2020						
Total	6047		6047		10127	

Source: [13]

The low responsiveness of HIV cases to the various social interventions by the Jamaican government supports a call by Professor Figueroa to strengthen the national HIV programme [10, 14].

The Government of Jamaica responded by announcing a \$1 billion spending on HIV/AIDS response for the 2022/2023 fiscal year [11]. The reality is that many CSOs including the Jamaica AIDS Support for Life have been actively engaged in implementing social intervention programmes to address the HIV/AIDS epidemic. Professor Figueroa echoed the call: “Therefore the onus falls on HIV Program leaders to implement quality services more effectively including introducing PrEP [Pre-exposure prophylaxis]. We are confident that with the support of the government, and commitment of all stakeholders and partners, a strengthened HIV Programme can achieve HIV epidemic control by 2030” [14]. The recommendation of Professor Figueroa is on the heel of the Coronavirus disease 2019 (COVID-19) pandemic. Many Civil Society Organizations in Jamaica were actively engaged in social intervention programmes during the pandemic and had received funding from the Ministry of Health/Global Fund.

Other circumstances that impacted human existence and well-being during COVID-19

The Ministry of Health/Global Fund was earmarked for other circumstances that were occurring in Jamaica during COVID-19 that exclude people living with HIV/AIDS. Studies have been conducted during the COVID-19 pandemic which showed an increase in domestic violence (i.e., emotional, sexual and physical abuse), and other issues [15]. Bourne and colleagues [15] opined that Jamaican homes have become unsafe for women and children, and this concretizes a rationale for resources from the Ministry of Health/Global Fund to address challenges during COVID-19. Money was allotted and spent by nine CSOs in Jamaica, and the next phase of the process is the assessment.

No assessment of funding geared towards reducing vulnerabilities during COVID-19

With no assessment of the programmes implemented by the Civil Society Organizations (CSOs) there is no evidence of the effectiveness of their activities during the COVID-19 pandemic. A critical question that must be answered is ‘How did the Civil Society Organizations (CSOs) respond during the COVID-19 pandemic, and were their programmes effective in addressing the challenges of those living with HIV and preventing an increase in new infections?’ As a result, this study examined the six research questions. They are as follows:

- 1) How were the service delivery and well-being affected by the advent of the COVID-19 pandemic?
- 2) What was the degree of interference of COVID-19 on the CSOS’ service deliveries in Jamaica?
- 3) How was the COVID-19 response of Global Fund used to effectively assist in overcoming the disruption of the critical services provided by the CSOs?
- 4) Were the alternative methods introduced during the COVID-19 pandemic able to effectively meet the needs of clients?
- 5) How prepared were the CSOs for a pandemic prior to COVID-19?
- 6) How are policymakers and stakeholders to structure themselves in the future to deal with future pandemics?

Literature Review

Since the first confirmed case of Coronavirus disease (COVID-19), the world has experienced unprecedented retardation of physical mobility (i.e., lockdowns), an increase in public health practices such as sanitation and hand washing, and increased job placement. Initially, the Centers for Disease Control and Prevention (CDC) and the World Health Organization (WHO) were focused on containing COVID-19. During this time many social, physical, and emotional problems were brewing and had reached a stage of pending explosion. During the lockdown, many people were having sexual intercourse and there was a rise in rape and carnal abuse, domestic violence, and substance use. Furthermore, fewer people were using condoms; there was an increase in murder; a rise in the number of missing people, especially children; and an increase in alcohol consumptions. For this study, the researchers will be primarily concerned about the same affairs in Jamaica during 2020 and beyond, as well as some instances in other geopolitical zones across the world.

In a 2020 study, Bourne and colleagues [17] using divorce time series data from 2001 to 2020. found that there were 3,689 recorded legal separations in 2020 [17], which means that there were 10 divorces every day. Divorce is an obvious indication of irreconcilable challenges in Jamaican homes and may account for the rise in missing people during 2020 and 2021. The disintegration of the Jamaican family structure is evidenced in the number of children who go missing and then return yearly. Bourne and colleagues, using missing people data from the Jamaica Constabulary Force (JCF), found that one person goes missing every four hours in Jamaica; there are 75% of returned cases in 2020 and 55% in 2021, and 66% of cases are children [18]. Furthermore, in 2020,

22% of missing people were still unaccounted for and this increased to 47% in 2021 [18]. This begs the question, is the home a safe place during 2020 and 2021?

During the height of the COVID-19 pandemic in 2020, Bourne et al. [19] conducted a web-based cross-sectional national survey in Jamaica and found that: 1. 20% more people were engaged in sexual intercourse during the year, 2. 13% had at least two sexual partners, 3. 58.3% were consuming alcoholic beverages at home, 4. 66% were consuming alcoholic beverages, 5. 17% of those who were consuming alcoholic beverages had at least two sexual partners compared to 5.5% of those who did not consume alcohol. Jamaicans were not only consuming alcohol at home, but they were also engaged in risky sexual behaviour during the pandemic. Another study conducted by Bourne et al. [20] found that 38.2% of Jamaicans indicated that they were practising safe sex. Another study found that there was a 7.3% increase in Jamaicans having more sexual partners during COVID-19 and that there was an increase in sexual engagement [21]. Then there were many cases of domestic violence (32% of women during the pandemic) [22]. Of the number of domestic violent acts, 60.0% were physical, 31.1% were emotional abuse, and 8.9% were sexual abuse [22]. The unsafe sexual practices of Jamaicans warrant the question, were people mindful of HIV/AIDS during the pandemic? During the same period, there was an increase in sexual abuse cases, and fewer people using a condom, according to statistics from the Jamaica Constabulary Force.

Studies have been conducted outside of Jamaica and they found that sexual problems have increased during the COVID-19 pandemic [23, 27-30]. Globally, among the sexual problems that have increased during the pandemic is sexual exploitation/rape [31-34]. Harvard Medical School of Public Health opined, “Even without the added burden of a global pandemic, only 23% of sexual assaults are reported to the police.

In the cases that have been reported during the COVID-19 pandemic, victims are finding it increasingly difficult to access the resources they need” [33]. The matter highlighted by Harvard Medical School of Public Health speaks to the psychological effect of sexual exploitation and when this is coupled with the assailant not using a condom, the challenges associated with the pandemic have a silent operant –the sexual diseases that may occur during the rape, including HIV/AIDS infections due to the rise in rape cases. According to Brown and colleagues, “The ongoing collision of these two global pandemics will continue to need both study and interventions to mitigate the effects of COVID-19 on HIV efforts worldwide” [35].

HIV/AIDS

The world was ambushed by a new disease called Coronavirus 2019 (COVID-19), and many people wanted to ascertain how this would influence many of the existing health conditions and, by extension, life. Hence, one question that was on people’s minds was will COVID-19 influence patients with the HIV/AIDS virus [36]. This question stood at the forefront of people’s minds, and many stakeholders questioned how they were to carry out their responsibilities during this crisis. USAIDS provided critical information on its website entitled ‘COVID-19 and HIV’ to clarify issues for both the stakeholders and those living with HIV [37].

Recognizing the challenges of HIV/AIDS patients, the Regional Office of the World Health Organization in Africa [38] recommended that services should remain unchanged for this critical cohort. The Regional Office for Africa of the WHO made the following statement:

This year’s commemoration of World AIDS Day [2021] occurs in a complicated terrain where essential services have been disrupted by the COVID-19 pandemic. Under the theme "End Inequality, End AIDS. End pandemic" this day is more than a call for a balance in

health interventions to give all patients the chance to have equitable access to health services [38]

The WHO placed squarely on the table the need for the continuation of services for those living with HIV/AIDS. Those words echoed by the Regional Office for Africa of WHO highlights the need for the continued care of people living with HIV/AIDS. This call goes to the core of the care of those living with HIV/AIDS across the globe and provides a rationale for the global fund that was a response to COVID-19. Furthermore, studies have been published on HIV/AIDS and the authors indicated that the pandemic could provide a platform to guide a response to COVID-19 [39, 40]. Those studies will provide a base for the current work as well as a guide for designing a current crisis management framework for Jamaica.

The Ministry of Health received some funds from the Global Fund to fight AIDS in response to COVID-19 and it was distributed to many Civil Society Organizations (**CSOs**) as part of the Global Fund's response to COVID-19 [41]. According to the Global Fund,

The COVID-19 Response Mechanism (C19RM) has awarded US\$4.3 billion to 131 countries since April 2020, to mitigate the impact of the COVID-19 pandemic on programs to fight HIV, tuberculosis (TB) and malaria and initiate urgent improvements in formal and community health systems. Funding awarded through C19RM is available until 31 December 2023. Implementing countries are encouraged to look at opportunities for refocusing C19RM funding in line with evolving epidemiological country contexts [42].

The monies have been expended on various HIV/AIDS-related responses during COVID-19; but the reality is a lack of an assessment of the spending, whether the spending was effective in responding to COVID-19, and whether the response to COVID-19 can be used to guide crisis

response in the future. Those issues are unanswered, and accountability requires the assessment of response by JASL and CSOs in Jamaica. Hence, this study seeks to evaluate the cost-benefit analysis of the funds and to design a crisis management plan that can be applied in the future to address such challenges brought on by COVID-19.

Jamaica AIDS Support for Life (JASL)

Contemporary Jamaica has immensely evolved since the 1980s when the society recorded its first AIDS case. During the 1980s, there was high stigmatization and discrimination against those who were diagnosed with HIV/AIDS. This stigmatization and discrimination against HIV/AIDS patients mushroomed into the formation of the JASL [42]. The JASL emerged because of the psychological suffering of a person, and the concerned citizens sought to address the challenge that was inherent in a health care system [42]. Then, in 1991, the organization was the first non-governmental organization (NGO) that would respond to issues experienced by those diagnosed with HIV/AIDS as well as the education of the populace on the virus [42].

JASL was urged by the Ministry of Health to continue the work among persons living with HIV & AIDS (PLHIV), men who have sex with men (MSM) and sex workers (SW) as the trust had been established through the provision of a stigma-free zone in which everyone could access testing, counselling, and treatment regardless of sexual orientation, gender, race, occupation, colour, class economic status or religion. The JASL also serves transgender women (TGW), orphans and vulnerable children (OVC), the hearing impaired and deaf, and key groups of women who are the most at risk for violence (*Jamaica AIDS Support for Life*).

JASL is spearheaded by a Board of Directors with the day-to-day operations of the entity being managed by an Executive Director. The organization's mission is to be a world-class leader

in creating and utilizing best practices in the delivery of services to persons living with and affected by HIV and AIDS in Jamaica and participating in the fight against the spread of HIV and AIDS in Jamaica in an enabling environment. JASL's efforts are made possible through donations and two of their sponsors are The United Nations Democracy Fund (UNDEF) and the United States Agency for International Development (USAID) (*Jamaica AIDS Support for Life*).

Like other NGOs or community-based organizations (CBOs) or civil service organizations (CSOs) in Jamaica, JASL aims to foster the lowering of risk factors among those living with HIV/AIDS and to continue with the provision of services to their clients. During the COVID-19 pandemic, chief among the responsibilities of this organization was the provision of answers to their clients on some questions relating to COVID-19 and its influence on HIV/AIDS. Many of the answers to previous matters were obtained from the Centers for Disease Control and Prevention [43], PAHO [44], and WHO [45], and these were related to their clients. COVID-19 provided additional ongoing challenges faced by those living with HIV/AIDS. To address and respond to COVID-19, additional funding was needed, and this was made available to JASL by the Ministry of Health.

“Jamaica has benefited from a high level of commitment to HIV/AIDS from the Ministry of Health. Successes include the strengthened capacity to manage prevention, treatment, care and support and more recently, policy issues” [46] and this commitment extends to the allocation of new finances from the global funds for AIDS to respond to the HIV/AIDS during COVID-19. JASL was among the recipients of Global Funds for AIDS, but no assessment has been conducted on how this allocation benefits those living with HIV/AIDS during the pandemic.

According to the Ministry of Health, “HIV/AIDS has become a generalized epidemic in Jamaica that affects the health and well-being of large numbers of people from all social classes and occupational groups throughout the country. However, HIV/AIDS is not only a health problem; it is a developmental issue that affects the social, cultural, political and economic fabric of the nation” [46]. This provides the bedrock for sourcing and allocating resources to those living with HIV/AIDS as well as other vulnerable groups who would be experienced severe difficulties because of the COVID-19 pandemic [47]. Hence, the Global Fund was allocated to many CSOs in Jamaica to alleviate the challenges caused by the COVID-19 pandemic.

Community-based organizations (CBOs) or community service organizations (CSOs) are non-profit, non-governmental, or charitable organizations that represent community needs and work to help them. CBOs may be associated with a particular area of concern or segment of the community. There are numerous community-based organizations involved in all sectors of Jamaica's national development, notably in education, health, and the environment. Those CBOs/CSOs were engaged by the Ministry of Health and Wellness (MOHW) because they had apparatus that can reach the vulnerable groups. This study will examine those CBO/CSOs that were engaged by the MOHW to alleviate the stressors associated with the COVID-19 pandemic.

Jamaicans for Justice (JFJ)

Jamaicans for Justice (JFJ) is a non-governmental human rights and social justice organization [48]. JFJ was founded in 1999 in Kingston, Jamaica. The group was co-founded by Jamaican human rights activist Dr Carolyn Gomes who was awarded the United Nations Prize in the Field of Human Rights in 2008. JFJ believes in the protection of the Constitutional rights of every human being and their work is guided by recognition and acceptance of The Universal Declaration of Human Rights (UDHR). Their mission is to engender fundamental change in

Jamaica's judicial, economic, social, and political systems to improve Jamaicans' present and future lives. JFJ serves hundreds of Jamaicans each year by providing legal services in response to human rights violations, working on legislation and policy, campaigning for social justice causes, and conducting high-impact research that shapes the national human rights agenda [48, 49].

Jamaicans For Justice (JFJ) provides direct legal support in human rights matters which involves legal assistance to victims of human rights violations and their families, pro bono legal advice to primarily low-income and marginalized persons, and strategic litigation of human rights, social justice, and state accountability matters [48, 49]. Their legal team works primarily on cases involving public authorities, especially the security forces and they provide legal support in response to police killings, unlawful detentions and abuse, discrimination of any kind, denials of access to justice and other forms of injustice. The organization also provides outreach, education, special projects, policy, and advocacy in which they maximize public engagement in the policy-making process and justice services for victims of gender-based violence [48]. Their work is made possible through a combination of donations and grant-funded projects from various international development partners, foundations, and other partners. JFJ does not accept contributions from political parties. Some of their major sponsors include Amnesty International (US and JA), UNICEF, The United Nations Democracy Fund (UNDEF), American Friends of Jamaica (AFJ) and The Global Fund (*Jamaicans for Justice* 2021).

Jamaican Network for Seropositives (JN+)

The Jamaican Network of Seropositives (JN+) was established to advocate for the rights and concerns of people living with and affected by HIV and AIDS, through empowerment, partnership, and resource mobilization [50]. The Network began in 1996 and has managed to build

and maintain relationships with the Government of Jamaica, Civil Society organizations, private sector organizations and Multilateral Organizations. Some of their partners include The Joint United Nations Program on HIV/AIDS (UNAIDS), The Global Fund, the Ministry of Health, the National Family Planning Board, the Ministry of Labor and Social Security, Jamaica AIDS Support for Life, The Jamaica Community of Positive Women, Jamaica Red Cross, Caribbean Vulnerable Communities Coalition and NCB Foundation (*Jamaican Network of Seropositives* 2019).

JN+ provides support to the community of People living with HIV through capacity-building projects/programs using their Positive Health Dignity Prevention curriculum (PHDP). The organization receives complaints of HIV-related stigma and discrimination and other human rights violations against persons living with HIV. They are also engaged in other activities including:

- Coordinate and support the implementation of PLHIV self-support groups
- Conduct sensitization sessions with service providers to improve service delivery and the care and support to People Living with HIV
- Providing educational and employment opportunities through referrals and partnerships [50].

Jamaica Community of Positive Women (JCW+)

The Jamaica Community of Positive Women (JCW+) was established in March 2010 and is Jamaica's premier organization of its kind, to address the priority needs of women living with HIV [51-53]. JCW+ is a part of the International Community of Women Living with HIV/AIDS (ICW) and exists to enable these women to reach their full potential and be recognized as productive citizens [52]. One of its major roles as advocates is that of involving women in the decision-making

process of policies and programs that affect their lives, one of which is the Sexual Offences Act. At JCW+, they have documented the experiences that women of all ages encounter with domestic violence and one of the major expressions of violence against women surrounds sexual offences [51, 53].

For the period 2014 - 2018 JCW+ focused on leadership development, systems strengthening, advocacy related to sexual reproductive health and rights, ending violence against women, and building partnerships for reach and sustainability. An action plan developed in 2018, allowed participants to propose areas for focus and collaboration for further growth in 2019. Key advocacy areas identified included, Sexual and Reproductive Health Rights, Anti-Discrimination Laws, and Gender Based Violence [54]. Two of their contributors include UNAIDS and The Global Fund.

TransWave Jamaica

Established in 2015, TransWave Jamaica is the country's first non-profit organization solely dedicated to focusing on promoting the health and well-being of the transgender, gender non-conforming and intersex communities [55]. They started as an advocacy initiative in the form of a blog which was formulated by their co-founders Akilah White and Neish McLean during a workshop hosted by WE-Change Jamaica and partners. Their advocacy efforts are centred on key areas which promote the inclusion and protection of the transgender and gender non-conforming community. These include legal gender recognition, equitable access to non-discriminatory health care, education, housing, and employment [56].

TransWave takes a holistic approach to advocacy, addressing not only the socio-political issues faced by the trans community, but also the mental, emotional, and physical health through our various initiatives. TransWave Jamaica strongly believes that one's gender identity should have no bearing on whether someone can fully enjoy fundamental rights, like the right to be recognized by her/his/their government or to access health care, education, or employment. Their efforts are made possible through donors and grant partners which include the International Trans Fund, Astrea Lesbian Foundation for Justice, UNAIDS, Caribbean Vulnerable Communities, The Global Fund for Women and The Global Fund for AIDS, TB, and Malaria [56].

Equality for All Foundation (EFAF)

Equality For All Foundation (EFAF) is a human rights and social justice organization that advocates for the rights, livelihood, and well-being of lesbian, gay, bisexual, and transgender (LGBT) people in Jamaica [57]. Their work seeks to build a Jamaican society that respects and protects the rights of everyone, and they are committed to promoting social change, empowering the LGBT community, and building tolerance for and acceptance of LGBT people. EFAF envisions a Jamaica where LGBT citizens feel safe, secure, included, and inclined to settle. To this end, the organization's work is guided by the pursuit of five main outcomes:

1. Improved enabling environment for the livelihood, health, and well-being of LGBT Jamaicans.
2. Inclusion of the voices of the Jamaican LGBT community in policy and decision-making.
3. The provision and delivery of services that meet the needs of the community.
4. Increased opportunities for LGBT citizens to be empowered to access services.
5. Provision of human rights and legal literacy programs which empower LGBT citizens.

EFAF works in close partnership with TransWave Jamaica and Equality Youth Jamaica (EYJ), who share the same target audience, i.e., members of the LGBT community. EFAF's efforts are made possible through donations and sponsors. A few of their contributors include The Astraea Lesbian Foundation, Comic Relief/Impact, GiveOut, The Joint United Nations Program on HIV and AIDS (UNAIDS), The Commonwealth Equality Network, The European Union, The Global Fund and The United States Agency for International Development (USAID) [58].

Eve for Life

EVE for Life is an organization founded in 2008 that provides support to adolescent girls and young women who are survivors of physical and sexual abuse, and to women and children living with or affected by HIV and AIDS. It was later registered in February 2009 as a non-governmental organization (NGO) with charitable status. The organization's work is grounded on extensive research which informs their advocacy and service delivery to address the needs of their beneficiaries. Their mission is to improve the sexual and reproductive health and rights and quality of life of young women and girls exposed to HIV and sexual and gender-based violence [59].

EVE for Life runs a program that supports the social and psychological development of children living with or affected by HIV and AIDS. This involves counselling the children and their guardians and supporting them through providing basic education and life skills training that enable them to live meaningful lives. They also provide Voluntary Counselling and Testing Services (VCT), and Training to individuals and organizations to ensure the effective provision of counselling and support and mentorship to newly diagnosed women and young girls through the mentor mom and life coach services. EVE For Life has no state funding and less than 1% of its funding comes from local donors in Jamaica. Two of their sponsors include The Global Fund and UNICEF [59].

The ASHE Company

The Ashe Company was founded in 1993 [60]. It is a non-governmental, internationally acclaimed edutainment and theatre arts company in the English-speaking Caribbean. The entity is committed to the preservation and renewal of the Jamaican/Afro-Caribbean culture using “Edutainment” (educating while entertaining), entertainment, community transformation, youth empowerment and social development projects. Ashe consists of a full-time cast of professional artists who provide entertainment and edutainment presentation services, locally and around the world. Ashe, a Yoruba word used in Western African spirituality, literally means "the strength, the power and the energy within." They believe that every individual has within them the capacity to develop and use their inner gifts and talents to live a better life. This puts the responsibility of living the life we love on ourselves. Ashe's goal is to be one of the pathways to assist people, to transform our world by transforming themselves first [60, 61].

Their services include entertainment and edutainment performances and production, jingles, video content creation, and live streaming, as well as health support services such as community interventions, free testing for HIV, and workshops on personal development. One of their social development projects includes the 2022 Campaign "I LIVE SAFE ALL THE WAY." This campaign focused on encouraging individuals to get vaccinated, focusing on the facts and myths of living healthy and maintaining healthy practices in and out of the bedroom, and teaching and reminding the population of effective condom use. Their initiative has created a partnership with the Ministry of Health and Wellness and is funded by international donors such as the U.S. Agency for International Development (USAID) and the Global Fund.

Children First

Children First is a non-governmental agency that caters to the needs of children through the provision of education [62]. Based in Spanish Town, Jamaica, Children First is Jamaica's largest program of this type, offering social, educational, and training programs for youngsters in the 10-24 age group while empowering their parents and guardians to overcome poverty and enhance daily life through successful skills training and small business projects. Children First has been recognized at home and abroad for its creative, participatory approach, which ensures its young client's full involvement including decision-making at the board of directors' level. The mission of Children First is to work with the street, working and vulnerable children, to improve their life opportunities and enable their contribution to society, through active engagement with children and young people, their families, communities, and institutions which affect their lives and their vision is a Jamaica in which children are active participants in a country which respects, protects, and fulfils their rights to develop to their full potential, free from abuse and neglect. Two of its programs include:

- I. The Children First Youth Wellness Center (YWC) is a response to two critical challenges facing young people in Jamaica; their vulnerability to HIV/AIDS infection as shown by an increased infection rate among adolescents; and the reluctance of youth to use national health care facilities because they are insensitive to their needs. The Center provides a unique combination of services aimed at equipping each young person with the information, skills and health care needed to combat the risk of contracting HIV/AIDS. The Youth Wellness Center (YWC) offers education and vocational skills training, job placement opportunities and user-friendly support services [62].

II. The Bashy Bus Kru which was pioneered as Jamaica's first mobile reproductive health and information service is geared at empowering young people to make healthy life choices. The Bashy Bus Kru provides adolescents with easily accessible, youth-friendly health services in a bid to reduce the spread of HIV/AIDS and other Sexually Transmitted Infections in Jamaica. It is equipped with an examination room as well as multimedia and audio equipment to appeal to its target audience. Health screening, information, contraceptives, and psychosocial services are provided by a multidisciplinary team of outreach workers, social workers, psychologists, and other trained healthcare professionals, whilst maintaining a standard of quality and confidentiality. Notably, clientele that is deemed most at risk i.e., key populations, including persons living with HIV is catered to via the linkage to care, treatment, and support/case management services [62].

The organization is funded and supported by The United States Agency for International Development (USAID), The Global Fund, Grand Challenges Canada, Heart Trust NTA and the C.B. Facey Foundation [62]. With the onset of COVID-19, people in vulnerable groups including HIV/AIDS patients were experiencing severe psycho-socioeconomic challenges and The Global Fund provided needed financial resources to mitigate some of the effects of the pandemic. This research seeks to examine the experiences, challenges, and resource utilization of the funds accessed through the Ministry of Health and Wellness, and how funds aided the well-being of the intended population. To provide a comprehensive assessment of the accessed funds, this study used a mixed research perspective (i.e., survey research, elite interviews, and focus group) to examine the objectives.

Methods and Materials

For us to get a comprehensive understanding of any phenomenon one must examine it from different perspectives, and this must be done by way of more than one method/approach. A hybrid research approach offers more truths. The hybrid approach is done by twinning methods, which allows for triangulation truths and provides in-depth knowledge of the studied phenomenon. With the hybrid approach providing more in-depth knowledge of a phenomenon, this research employed both an objective (i.e., quantitative) and a subjective (i.e., qualitative) approach to assess the services delivered by selected community service organizations (CSOs) that received funding from the Ministry of Health and Wellness that is made possible from the Global Fund to aid vulnerable groups during the COVID-19 pandemic.

Quantitative perspective (survey research)

For the quantitative perspective, a web-based non-probability sampling cross-sectional descriptive research design via a survey was employed [62-66]. Two standardized instruments were designed (i.e., client and CSOs). The client's instrument was a 22-item standardized item (5 open-ended and 17 close-ended items; with there being five demographic items; See Appendix I). A 26-item standardized instrument was designed to collect data from members of the CSO (3 open-ended and no close-ended items; See Appendix II).

The data were collected over a three-week period from October 20, 2022, to November 5, 2022. The instrument was designed using textbooks on survey research and designing surveys [64, 66], and the items were in keeping with the research objective (i.e., assessing the effectiveness of utilizing funds from the Ministry of Health and Wellness to aid selected vulnerable groups during the COVID-19 pandemic).

The researchers sought and protected the identity of the respondents by

1. Not using any personal identifier except age group, members, educational attainment, area of residence, nationality.

2. Informing respondents that they may withdraw at any time during the process and return their questionnaire without any form of penalization or repercussion.

3. Collecting the data by way of the internet. The instrument was designed and later forwarded to the email addresses of current clients of the studied CSOs in Jamaica.

Each respondent's consent was based on the completion and submission of the instrument. The researchers instituted protection that would ensure that only one person can complete a questionnaire from an IP address.

Survey Monkey was used to collect the data, which allowed for the collection of web-based data and the provision of the Statistical Package for the Social Sciences (SPSS) database. The researcher downloaded the data to the SPSS database, where analyses facilitated the answers to the various research questions and objectives using descriptive statistics and percentages [63, 67]. The findings, therefore, were displayed using graphs and tables. Cross-tabulations were used to examine the bivariate analyses of non-metric variables (i.e., nominal and ordinal) – [63, 67]. A p-value per cent was used to determine the level of significance in a two-tailed test.

The researchers addressed the treatment of missing data in the current study by reviewing the extensive work on missing data to ensure alignment with recommended best practices [68-71]; there is no consensus on a rule of thumb on the matter. Hence, the researchers opted to employ two guidelines. These were 1. in the event there are more than 30% of the data is missing, the variable will be excluded from analysis, and 2. where there are less than 30% of missing cases, the system missing in the SPSS program will be used to assign a value for those cases.

Qualitative perspective

Three primary instruments were used for data collection for the qualitative aspect of the study; *elite interviews*, *focus groups* and *document review* [72-75]. The sources for data collection were conveniently divided into three areas—the Ministry of Health and Wellness, executives of the CSOs and Caribbean Vulnerable Communities.

Elite interviews

Semi-structured interviews were chosen as they were closest to the unstructured interview which is *flexible*, *iterative* and *continuous* [76] as well as more likely to yield information that was not planned for [77]. Semi-structured interviews allow for systematic consistency while giving sufficient latitude for the subject to ‘digress’ thus enabling a deeper probe [72] and facilitating new and unexpected information. The use of a semi-structured (instead of structured) format study enabled the researchers to make a deeper probe into the usage of the funds received, used, and benefits obtained by the recipients.

Elite interviews are most effective in obtaining information about decision-making and the decision-making process [78] as the subjects may be treated as ‘experts’. The interviews took the form of a “*guided conversation*” [76, 79] where the interviewees were seen not as “*passive conduits for retrieving information*”, but more for *interpretation* and *perspective* (ibid) thus facilitating a deep probe. The questions for the elite interviews were guided by the literature review and pre-existing notions of the researcher.

The elite interviews from the executives of the CSOs, the Ministry of Health and Wellness, and the Caribbean Vulnerable Communities sought to provide in-depth information on the studied phenomenon. The interviews also examined the extent to which these officials believed that such

emphases when articulated were replicated at the street level especially given the discretion of street-level personnel. A major focus of the interview was to evaluate the purpose of the funding, and the goals of the funding agency, and assess the deliverables of the programmes. All interviews were recorded (with the consent of the interviewee) and written notes were taken. The data gathered from the elite interviews are analysed by way of themes and narrations.

Results

Socio-demographic Characteristics of the Sampled Clients

Table 3 presents the demographic characteristics of the sampled clients. Of the sampled clients (n=80), the majority were females (75.0%, n=60), having secondary education (60.7%, n=48), were single (55.1%, n=43), age (31.6%, n=25), dwelling in St. Ann (31.3%, n=25). Table

3

Demographic Characteristics of the Sampled Clients

Details	% (n)
Gender	
Male	18.7 (15)
Female	75.0 (60)
Other	6.3 (5)
Educational level	
None	5.1 (4)
Primary	11.4 (9)
Secondary/high	60.7 (48)
Tertiary (College/university)	22.8 (18)
Current union status	
Single	55.1 (43)
Common-law	23.1 (18)
Married	14.1 (11)
Separated but not divorce	5.1 (4)
Divorced	1.3 (1)
Widow	1.3 (1)
Age cohort	
18-24	7.6 (6)
25-34	31.6 (25)
35-44	30.4 (24)
45-54	25.3 (20)
55-64	5.1 (4)
Area of residence	
Kingston	13.8 (11)
St. Andrew	8.8 (7)
St. Thomas	1.3 (1)
Portland	1.3 (1)

St. Mary	7.5 (6)
St. Ann	31.3 (25)
Trelawny	1.3 (1)
St. James	11.3 (9)
Hanover	1.3 (1)
Westmoreland	1.3 (1)
Clarendon	5.0 (4)
St. Catherine	16.3 (13)

Were the measures introduced during the COVID-19 pandemic able to effectively meet the needs of clients?

Figure 1 depicts a bar graph showing the sampled clients' response to receiving support from a CSO during the COVID-19 pandemic. Of the 80 sampled clients, the response rate to the question 'Did you receive support from a CSO during the pandemic?' was 97.5 per cent (n=78). Of the 78 clients, 91.03 per cent indicated that they have received support during the COVID-19 pandemic.

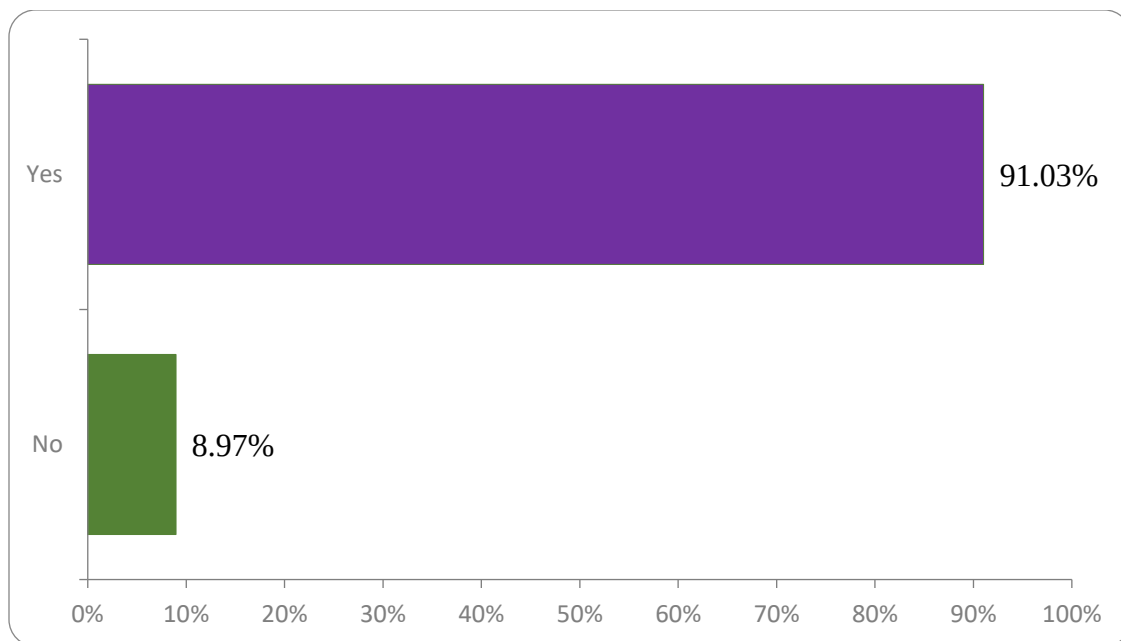


Figure 1: Received support from a CSO during the pandemic.

The sampled clients were asked an open-ended question, where did you receive the support, and 87.5% (n=70) of them indicate from whom the support was had (Table 4). The Clients received support from different CSOs during the COVID-19 pandemic.

Table 4

Source of support

Details	% (n)
Beth Jacobs	1.3 (1)
Children First	3.8 (3)
JASL	56.3 (45)
JASL & ASHE	1.3 (1)
JASL & EVE	1.3 (1)
JASL & JCW	1.3 (1)
JASL, JN, Children First, ASHE, & Equality for All	1.3 (1)
JASL, JN+, EFL	1.3 (1)
JCW+	13.8 (11)
Khalil and Karim Rose	1.3 (1)
Larry Chang Foundation	2.5 (2)
Ministry of Labour	1.3 (1)
St. Ann Health Centre	1.3 (1)
No response	12.5 (10)
Total	80

The sampled clients underwent many challenges during the COVID-19 pandemic. Most of the sampled clients underwent financial challenges (82.5%), and only 50% experienced health challenges. However, 80% of the sampled clients indicated that they obtained assistance from the CSOs during the pandemic.

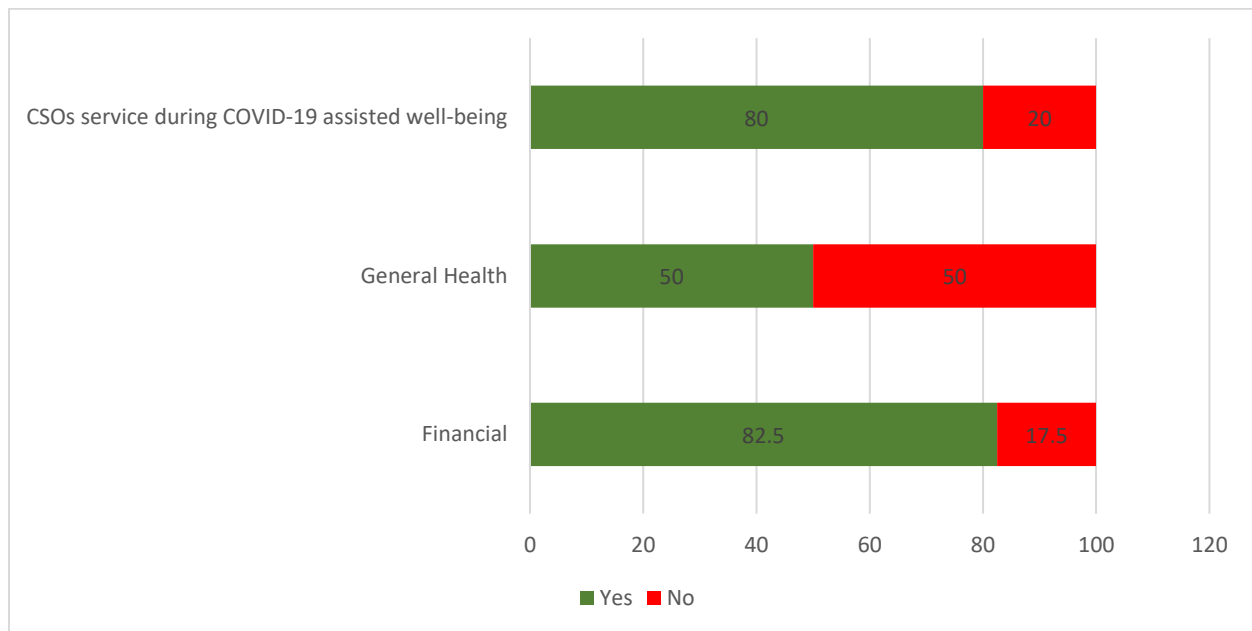


Figure 2: Challenges experienced during COVID and CSO assistance

Table 5 presents the views of the clients on the support received during the pandemic from a CSO. This was an open-ended question, and the indications of the respondents are captured in Table 5. The respondents gave single ideas as well as various combinations of support they obtained from a CSO. The sampled clients listed the type of assistance they received from a Civil Society Organisation during the COVID-19 pandemic (Table 5). Of the sampled clients, 11.3 per cent (n=9) of them did not indicate the type of aid they received from a CSO. The assistance varied from medical assistance, care package, educational grant, counselling to financial aid.

Table 5: Support received from the CSOs		
Details	Clients (n)	% (n)
Care Package	36	45.0
Care Package and Counseling	4	5.0
Care Package and Educational Grant	1	1.2
Care Package and Medical Care	1	1.3
Care Package, Financial, and Counseling	1	1.3
Counseling	4	5.0
Counseling and Care Package	2	2.5
Counseling and Social support	1	1.3
Educational/back-to-school support	3	3.8
Financial and Care Package	8	10.0
Financial grant	1	1.3
Financial, Care Package, and Counseling	1	1.3
Financial, Counseling, and Care Package	1	1.3
Medical Care	4	5.0
Medical Care and Educational Grant	1	1.3
Medical, Care Package, and Educational Grant	1	1.3
Medical, Educational, and Care Package	1	1.3
No response	9	11.3
Total	80	100%

The sampled clients were asked to rate the customer service experience received from the CSO before the COVID-19 pandemic, and their responses are shown in Figure 3. Fifty-six and four-tenths per cent of the sampled clients (n=44) rated the services received from the CSOs before the COVID-19 pandemic as excellent compared to 15.4% (12) who said very good. Only 2.6% of the sampled clients indicated that the service received was fair (n=2), and 3.8% (n=3) stated very poorly. This means that 71.79% stated that the service delivery was at least very good compared to 6.41% who stated, at most, fair.

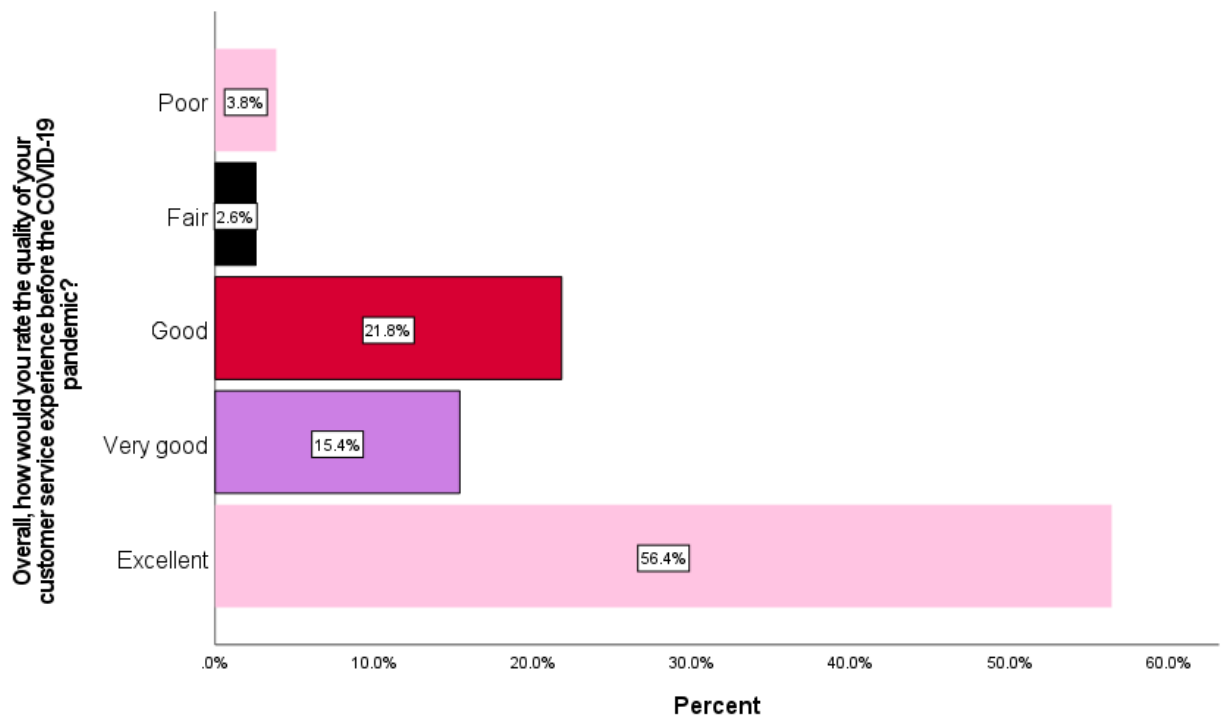


Figure 3: Rating the overall customer service experience received from CSO before the pandemic.

Most of the sampled clients indicated that they received at least good service from the CSOs during the COVID-19 pandemic (83.53%, n=66; Figure 4), with 38% stating that this was excellent.

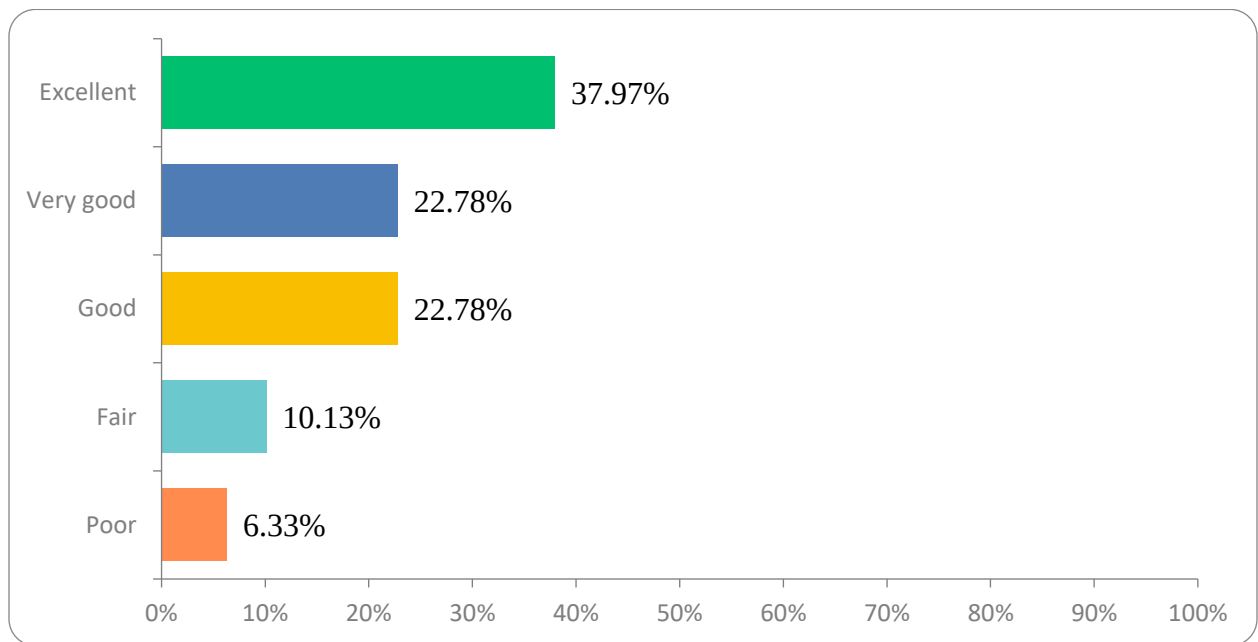


Figure 4: Rating the overall customer service received from CSO during the pandemic.

Table 6 presents a summary of the sampled clients' perspectives on the rating of the services they received from the CSOs before and during the COVID-19 pandemic. The sampled clients indicated the quality of the services received from the CSOs during COVID-19 has fallen (i.e., 32.6%; Table 6), and that poor service has increased by 65.8%.

Table 6

Summary of the rating of the services offered by CSOs

Detail	Before COVID	During COVID	% Change
	% (n)	% (n)	
Excellent	56.4 (44)	38.0 (30)	-32.62
Very good	15.4 (12)	22.8 (18)	48.05
Good	21.8 (17)	22.8 (18)	4.59
Fair	2.6 (2)	10.1 (8)	288.46
Poor	3.8 (3)	6.3 (5)	65.79
Total	78	79	

The sampled respondents indicated many challenges during the COVID-19 pandemic (Figure 5), with most of them stating financial difficulties (82.5%) followed by well-being challenges (80.0%).

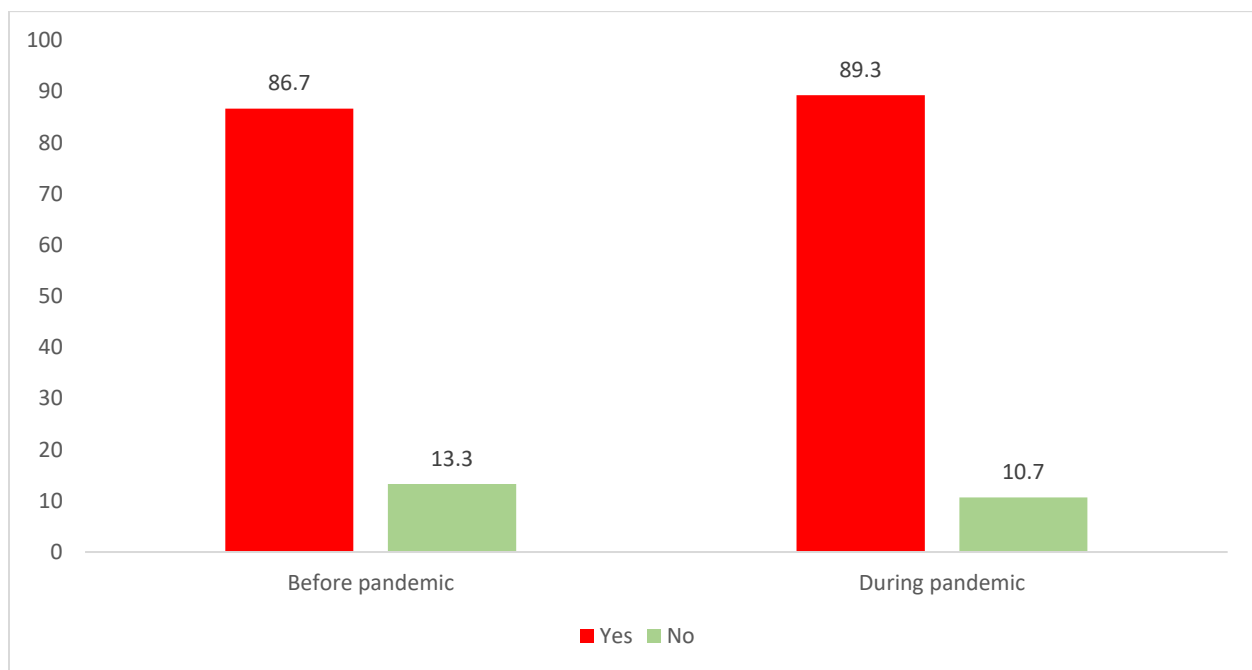


Figure 5: Challenges experienced during the COVID-19 pandemic

All the CSOs indicated they sought to adequately meet the needs of their clients as they are accustomed. The new measures that were implemented sought to address the challenges of the clients and these were in keeping with maintaining well-being. Some clients may have believed that more socio-economic support could have been provided by the CSOs, and so the measures were not totally effective in meeting their needs. There were clients who wanted to access resources from various agencies, and they were stopped because of the established protocol by the CSOs. Mr. Ivan Cruickshank of the Jamaica Country Coordinating Mechanism (JCCM)

referred to this as the 'Anansi culture' in Jamaica. The 'Anansi culture' is one in which people seek to trick others, scheme for resources, and use whatever means to acquire things.

How were the service delivery and well-being affected by the advent of the COVID-19 pandemic?

Figure 6 depicts the sampled clients' responses to whether there was a break in service delivery by the CSOs during the COVID-19 pandemic. Of the sampled clients (n=80), 96.25% (n=77) responded to the question 'Was there a break in services you received from the CSOs during the COVID-19 pandemic?' Fifty-six per cent of the sampled clients indicated that there was no break in service received from their CSOs during the COVID-19 pandemic. On the other hand, 4 in every 10 of the sampled clients indicated a break in service delivered by their CSOs during the COVID-19 pandemic.

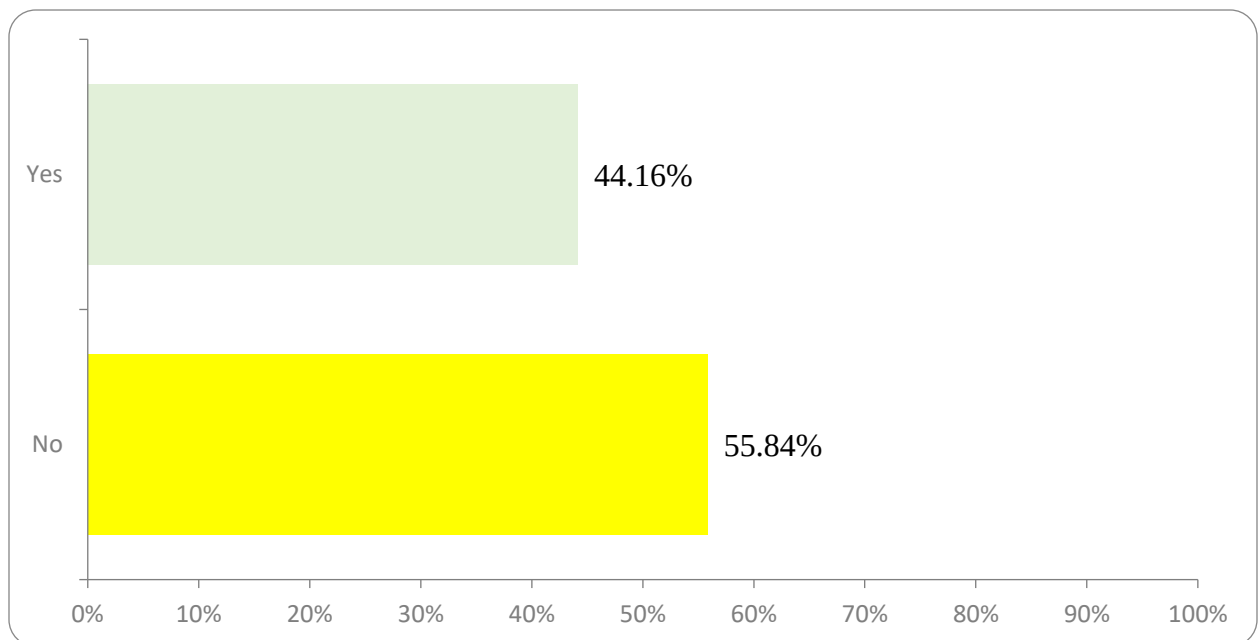


Figure 6: A break in services offered by CSO during the pandemic.

Figure 7 depicts the perception of the sampled clients on whether they are pleased with the quality of services received from the CSOs before and during the COVID-19 pandemic. There is a high level of satisfaction with the degree of services received from the CSOs both before (86.7%) and during (89.3%) the pandemic, with marginally more clients being pleased with the service delivery during the pandemic compared to before the pandemic.

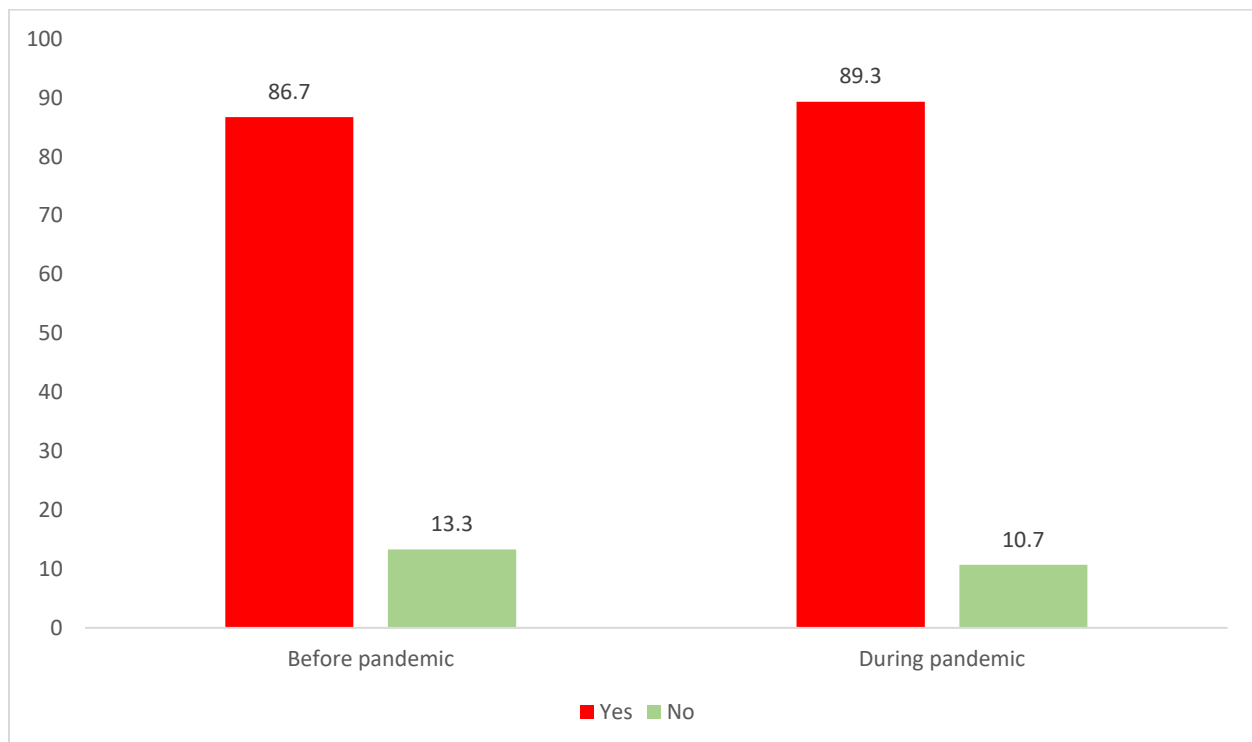


Figure 7: Pleased with the quality of services received from the CSO.

Figure 8 depicts a bar graph of the sampled clients' perspectives on their level of satisfaction with the service received from the CSOs during the COVID-19 pandemic. Most of the clients rated their level of satisfaction as 100% (17.1%) followed by 90% (12.9%) and 80% (12.9%). The pictorial display of the level of satisfaction of the clients speaks to a high degree of gratitude and fulfilment of needs by the CSOs during the pandemic, and pleasure derived from the work done by the CSOs during the pandemic. Overall, the average level of satisfaction with the services provided by the CSOs was 80% (range = 100%), with 68.6% rating their level of satisfaction to be at least 70%. Furthermore, 21.4% of the clients gave their level of satisfaction to be at most 40%.

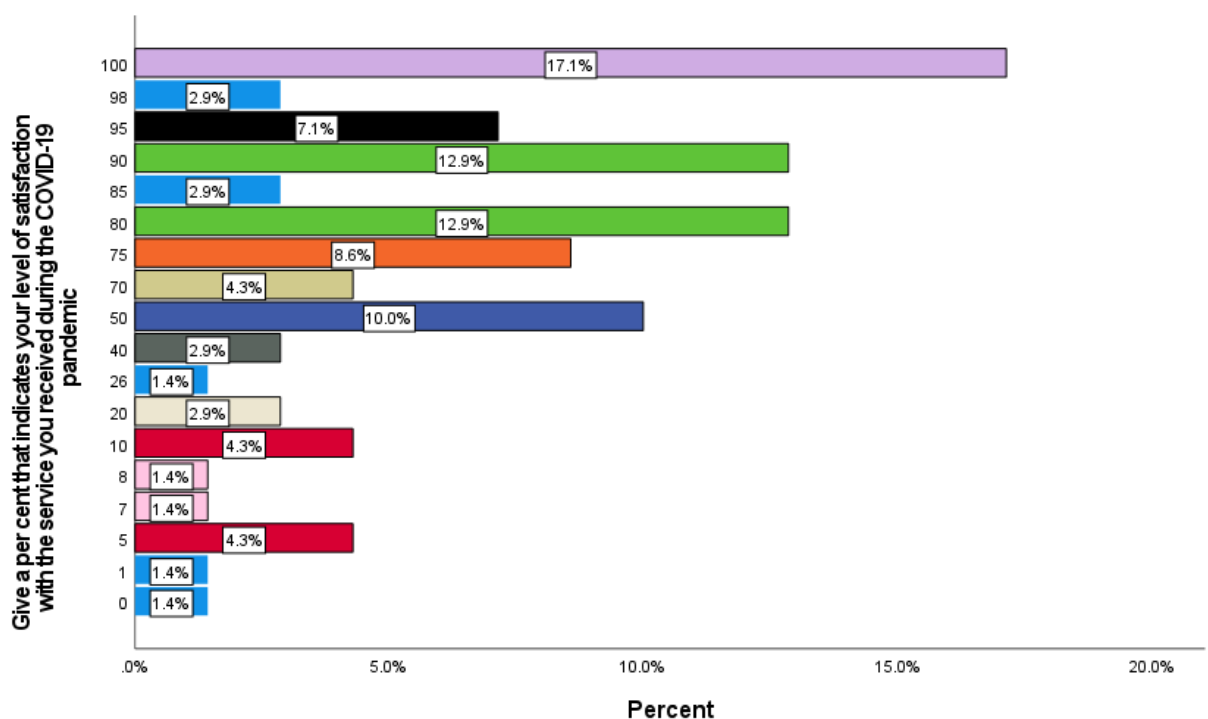


Figure 8: Rating the level of satisfaction with service received from the CSO during the pandemic.

Figure 9 is a box plot of the level of satisfaction of the clients with the work done by the CSOs during the pandemic. The box plot summarises the descriptive statistics of the level of satisfaction of the clients concerning the service rendered by the CSOs during the pandemic. From the box plot, on average, the clients rated the service done by the CSOs during the pandemic at 80%. The service rendered by the CSOs during the pandemic was widely felt by the clients, and this is expressed in the high average given by them.

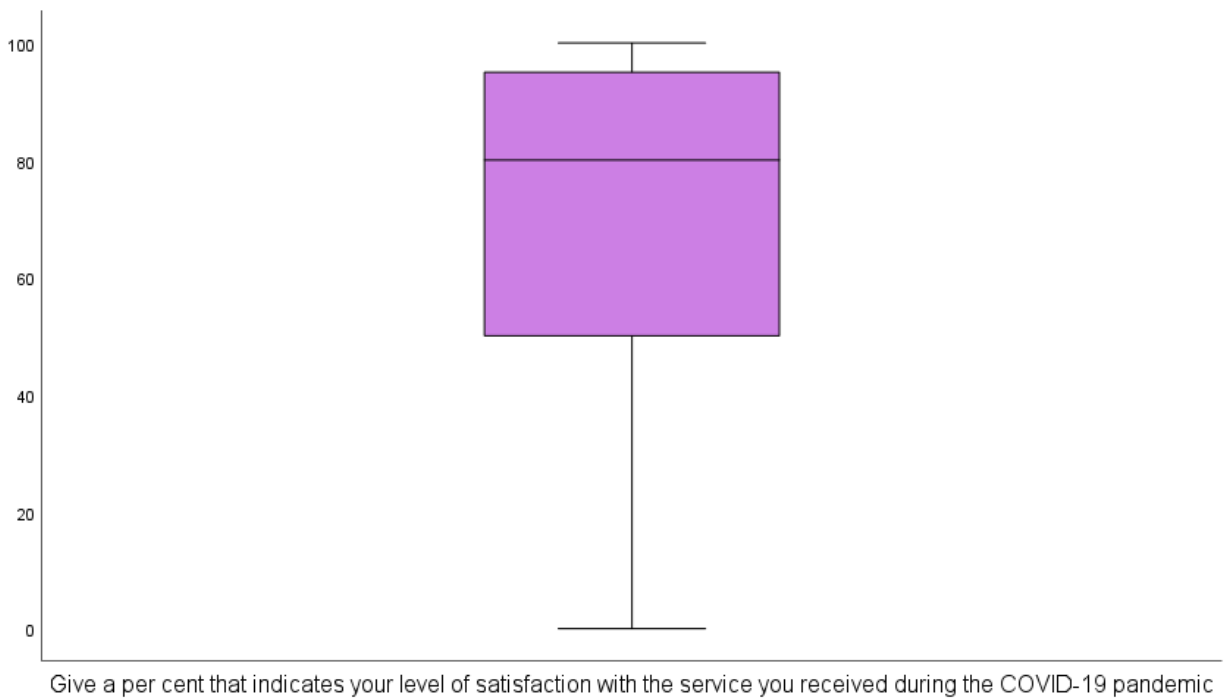


Figure 9: Rating the level of satisfaction with service received from the CSO during the pandemic.

The clients who rate the service they received from the CSOs as greater were more likely to express a greater level of satisfaction with the services rendered by the CSOs ($F [4, 85] = 10.760$, $P < 0.001$). This means that the greater the quality of services received by the clients, the more they are satisfied with the level of services received (Figure 10).

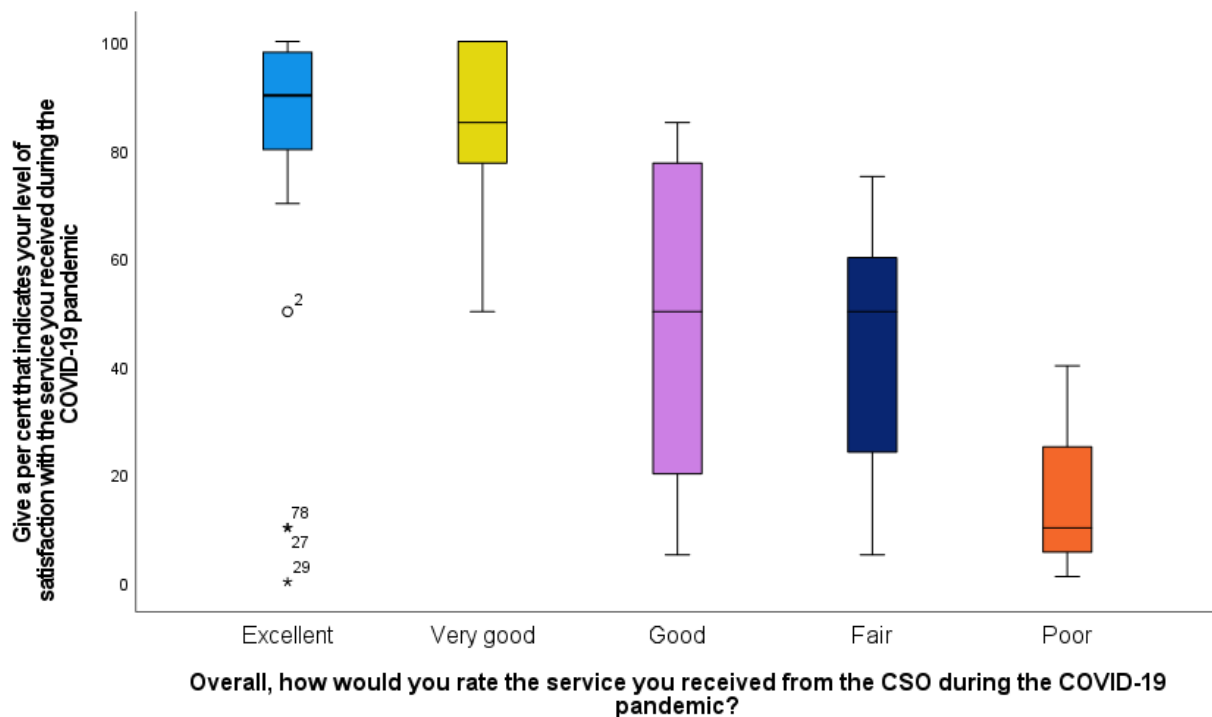


Figure 10: Rating the level of satisfaction with service received from the CSO during the pandemic by rating the service received.

The current self-reported health status of the sampled clients was good, and this is depicted in Figure 11. Eighty-eight per cent of the clients indicated that their self-reported health status was at least good, with 43.8% stating excellent health status.

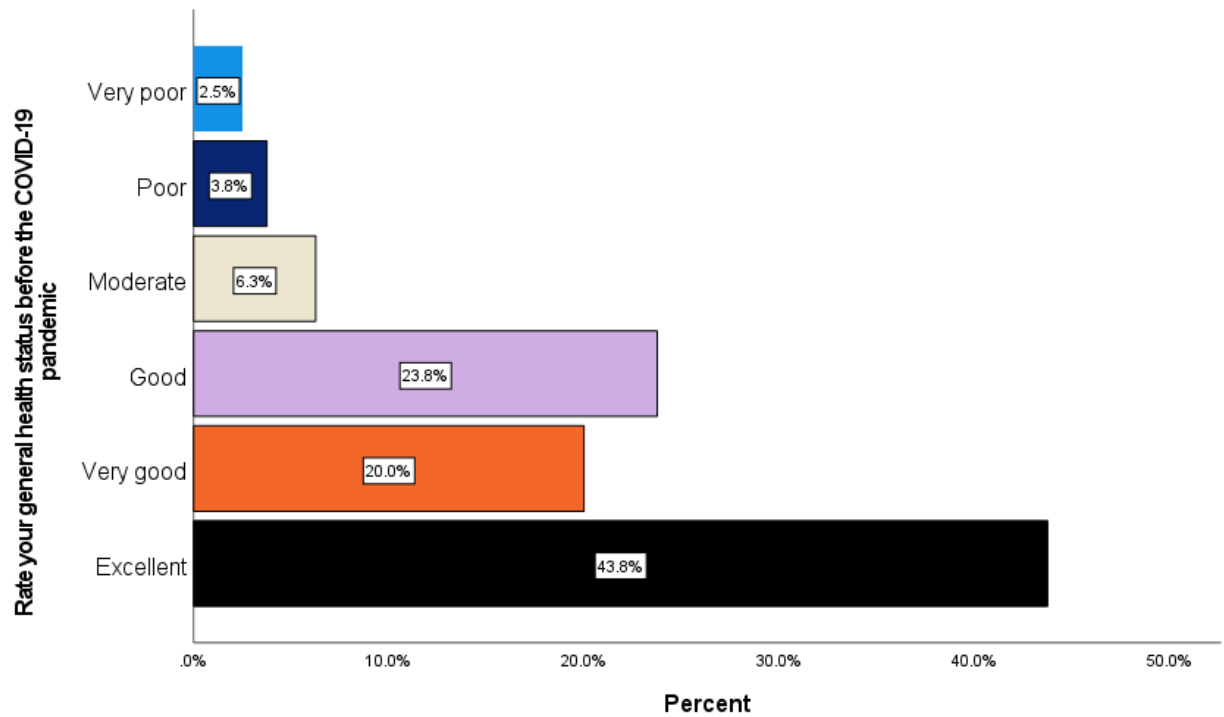


Figure 11: Self-reported health status

The sampled clients' level of satisfaction differs based on their health status ($F [5, 64] = 3.276, P = 0.011$). The box plot shows that clients with a greater self-reported health status indicated a higher degree of satisfaction with the services rendered by the CSOs (Figure 12). This means that the client's well-being during COVID-19 influences his/her level of satisfaction with the services rendered by the CSOs. It can be deduced from Figure 10 that if the services rendered by the CSOs contributed to the client's well-being, this means happier and more satisfied clients with the service provided during the pandemic.

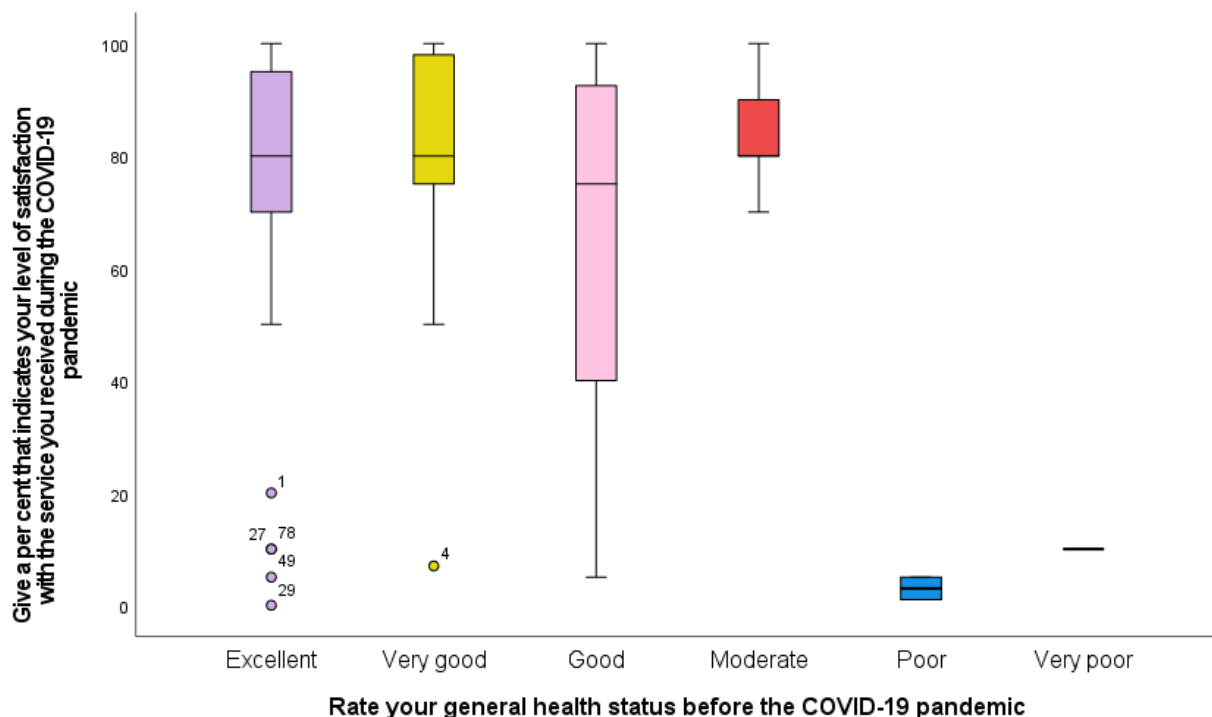


Figure 12: Self-reported health status by the level of satisfaction with service rendered by the CSOs.

Table 7 presents a cross-tabulation between rating the overall quality of customer service rendered by the CSOs before the COVID-19 pandemic and rating the general self-reported health status before the COVID-19 pandemic. Using the chi-square statistical test, a significant statistical relationship existed between the two variables (χ^2 (df=20) = 45.029, $P = 0.001$). None of the sampled clients who indicated a very poor self-reported health status rated the overall customer service quality offered by the CSOs before the pandemic as excellent compared to 73.5% (n=25) of those who rated their health as excellent.

Table 7

A cross-tabulation between rating the overall quality of customer service rendered by the CSOs before the COVID-19 pandemic and rating the general self-reported health status before the COVID-19 pandemic

Rating the overall quality of customer service rendered by the CSOs before the COVID-19 pandemic	Rate the general self-reported health status before the COVID-19 pandemic						Total
	Excellent	Very good	Good	Moderate	Poor	Very poor	
	% (n)	% (n)	% (n)	% (n)	% (n)	% (n)	% (n)
Excellent	73.5 (25)	50.0 (8)	42.1 (8)	40.0 (2)	50.0 (1)	0.0 (0)	56.4 (44)
Very good	14.7 (5)	18.8 (3)	15.8 (3)	0.0 (0)	0.0 (0)	50.0 (1)	15.4 (12)
Good	8.8 (3)	31.3 (5)	31.6 (6)	60.0 (3)	0.0 (0)	0.0 (0)	21.8 (17)
Fair	0.0 (0)	0.0 (0)	10.5 (2)	0.0 (0)	0.0 (0)	0.0 (0)	2.6 (2)
Poor	2.9 (1)	0.0 (0)	0.0 (0)	0.0 (0)	50.0 (1)	50.0 (1)	3.8 (3)
Total	34	16	19	5	2	2	78

Table 8 presents a summary of the sampled clients' perspectives on their self-reported health status before the COVID-19 pandemic and in the last 6 months. The sampled clients indicated excellent health status has declined by 20% and that their very poor health has declined by 52%. However, those who reported fair to very good health status indicated improvement and no change in health status was reported among those who stated poor health.

Table 8

Summary of self-reported health before COVID-19 and in the last 6 months

Detail	Before COVID	In the last 6 months	% Change
	% (n)	% (n)	
Excellent	43.8 (35)	35.0 (28)	-20.09
Very good	20.0 (16)	25.0 (20)	25.00
Good	23.8 (19)	26.3 (21)	10.50
Fair	6.3 (5)	8.8 (7)	39.68
Poor	3.8 (3)	3.8 (3)	0.00
Very poor	2.5 (2)	1.2 (1)	-52.00

Of those who indicated that their self-reported health status was excellent, 92.0% of them obtained services from the CSOs during the COVID-19 pandemic compared to 84.2% of those with very good self-reported health status, 71.4% of those with good self-reported health status, and 100% of those with very poor self-reported health status ($\chi^2(df=5)=11.996$, $P = 0.035$). Of importance is the fact that 8 in every 10 of the sampled clients indicated receiving service from a CSO during the COVID-19 pandemic.

Table 9

A cross-tabulation between received service from the CSOs before the COVID-19 pandemic and rating the general self-reported health status before the COVID-19 pandemic

Received services from CSOs during the pandemic	Self-reported general health status in the past six months						Total
	Excellent	Very good	Good	Moderate	Poor	Very poor	
	% (n)	% (n)	% (n)	% (n)	% (n)	% (n)	
Yes	92.0 (23)	84.2 (16)	71.4 (15)	71.4 (5)	0.0 (0)	100.0 (1)	80.0 (60)
No	8.0 (2)	15.8 (3)	28.6 (6)	28.6 (2)	100.0 (2)	0.0 (0)	20.0 (15)
Total	25	19	21	7	2	1	75

The congruence of the views that the COVID-19 pandemic was unplanned for were echoed by all the stakeholders including the vulnerable clients. The service delivery was unavoidably hampered, and this affected the clients. Dr Nicola Skyers of the Ministry of Health and Wellness provides some context for the increased demand for social support from people in Jamaica due to the disruption caused by the pandemic. She opined, “More persons were now asking for that kind of social support, which increased significantly during the pandemic. It was almost like a mechanism to encourage people to remain in care which is what they wanted to do. So, persons were coming in for the social support. Therefore, we were still able to engage them in the contact that they would normally need to get and they're coming to the clinic.” Undoubtedly, the pandemic had increased the expectations of the clients because they were faced with more challenges including survivability.

The disruption was for a few days and when the CSOs began responding to the new reality, this lessened the devastating effect of the pandemic. There is evidence from the current study that the pandemic caused massive disruption in social support services offered to clients, and that their well-being was negatively influenced during the early stage of the pandemic. Mr Cruickshank argued that the nature and lack of information on COVID-19 aided in the disruption of services offered by the CSOs, which was the same sentiment expressed by members of the CSOs.

Nevertheless, Mr. Cruickshank believed that the initial response by the government was a medical one and that it excluded the psychosocial aspect of people. The medical response to the COVID-19 pandemic, therefore, meant during the time that people were ordered to remain in their communities, many experienced lowered psychosocial well-being. The pandemic influenced well-being because needed services were not provided, and some people suffered socially and psychologically during the initial lockdown.

What was the degree of interference of COVID-19 on the CSOs' service deliveries in Jamaica?

Table 10 presents the sampled clients' rating of the level of disruption of service delivery caused by each of the following COVID-19 mitigation measures. Figure 13, on the other hand, depicts the ranking of the disruptions of service delivery based on high and very high levels. Hence, the clients indicated that mask-wearing was the most preventative measure in service delivery during the pandemic followed by social distancing, reduced opening hours, curfews, vaccination, and working from home.

Table 10
Rate of the level of disruption of service delivery caused by each of the following COVID-19 mitigation measures.

Details	1	2	3	4	5
	%	%	%	%	%
Curfew, restricted travel	12.8	5.1	52.6	20.5	9.0
Mask wearing	13.0	5.2	31.2	26.0	24.7
Reduced opening hours	13.0	10.4	46.8	23.4	6.5
Social distancing	8.0	10.7	41.3	16.0	24.0
Vaccination status	15.3	5.6	50.0	12.5	16.7
Working from home	23.6	8.3	44.4	13.9	9.7

Note: 1-Very low, 2 – Low, 3-Moderate 4 – High, and 5 – Very high

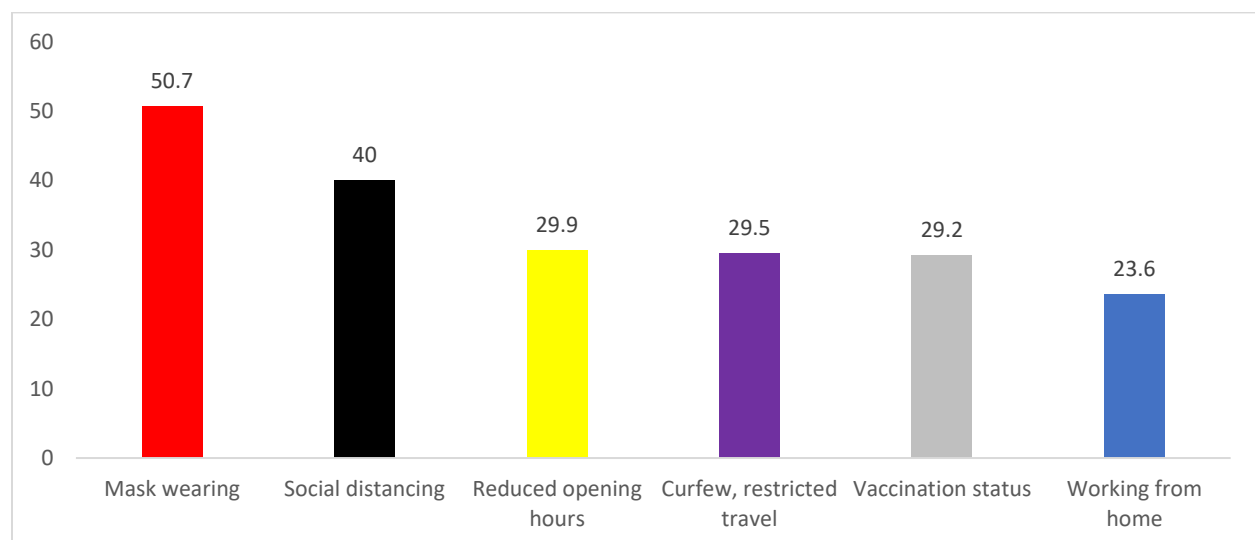


Figure 13: Rate of the level of disruption of service delivery caused by each of the following COVID-19 mitigation measures.

Table 11

Client's recommendations that could be implemented to improve the service delivery by the CSOs in the future.

Details
Design a food protocol
Offer more things to a household with families, especially those with HIV/AIDS
Design a communication protocol for reaching those in the vulnerable population
Leadership
Professional workers
Instituting a protocol that can reach the vulnerable population
Having a social media protocol
Designing a social interaction protocol including a hotline
Having a disability intervention protocol
Designing a mental health hotline for the vulnerable population
Designing a medical outreach protocol for the vulnerable population

Table 12 presents the responses of the CSOs officers on the effectiveness of their service delivery before the COVID-19 pandemic. Using high and very high (i.e., 4 and 5, respectively) to determine the effectiveness and very low and low (i.e., 1 and 2, respectively) to assess ineffectiveness, the officers of the CSOs believe that they have been effective in collaboration, communication and capacity building, collaboration, supporting infrastructure and sustainability of service delivery, and providing social and material support to their clients. However, the officers were indecisive on whether they have been effective in accessing justice and legal redress.

Table 12
The CSO rated the effectiveness of their service delivery **before** the COVID-19 pandemic.

Details	1	2	3	4	5
	%	%	%	%	%
Collaboration, Communication and Capacity Building	0/0	0.0	0.0	66.7	33.3
Supporting infrastructure and sustainability of service delivery	0.0	0.0	33.3	66.7	0.0
Providing Social and material support	0.0	0.0	33.3	33.3	33.4
Access to Justice and Legal Redress	0.0	0.0	66.7	33.3	0.0

Note: 1-Very low, 2 – low, 3-moderate 4 – high, and 5– very high

Figure 13 shows the officers of the CSOs' responses to the question, "During the pandemic, was there a break in your services?" They admitted that there was a break in their service delivery during the pandemic (33.3%, n=2). Despite the break in service delivery during the pandemic, many of them indicated that their services were continuous during the COVID-19 pandemic (66.7%, n=4).

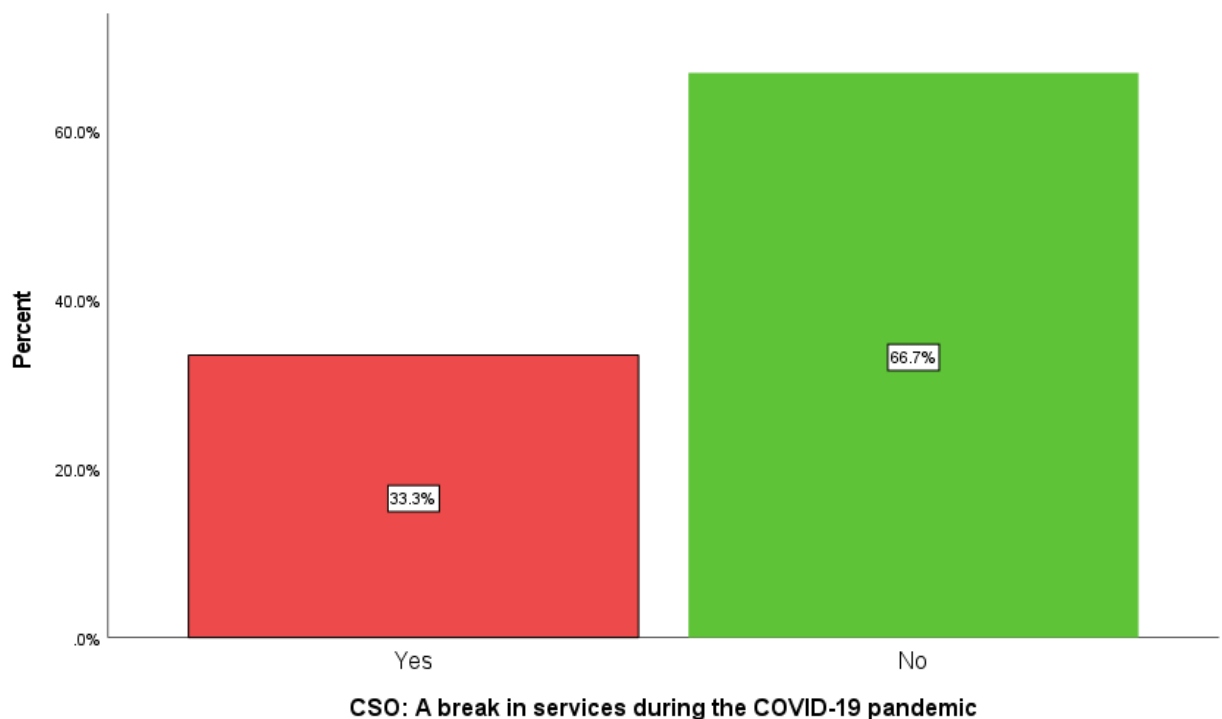


Figure 14: CSO: A break in services during the COVID-19 pandemic

The officers of the CSOs indicated that COVID-19 has significantly hampers the extent of their service delivery (67%, n=4), and only 33% (n=2) stated that did was not the case (see Figure 15).

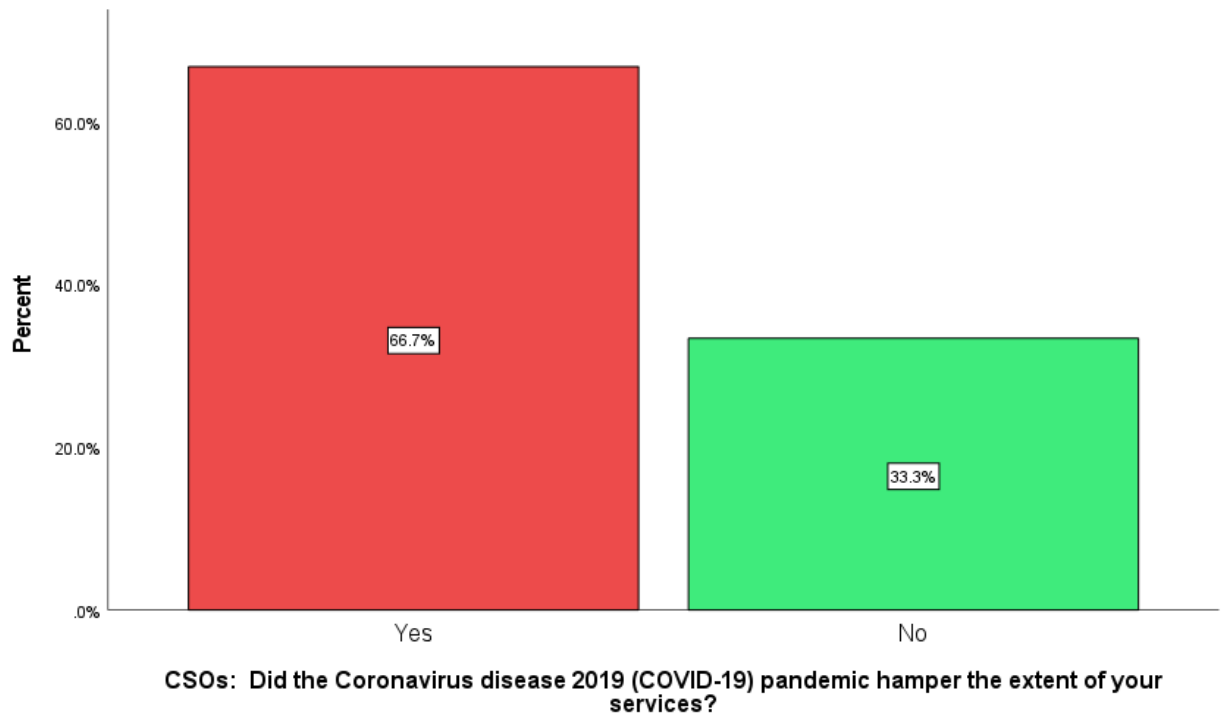


Figure 15: CSO: Did COVID-19 hamper the extent of your service delivery.

The officers of the CSOs were asked, “What aspect of your programme was most affected?” and they indicated three of the four options (i.e., prevention, treatment, and enabling environment). None of the officers indicated the rights of the clients were affected during the COVID-19 pandemic (Figure 16).

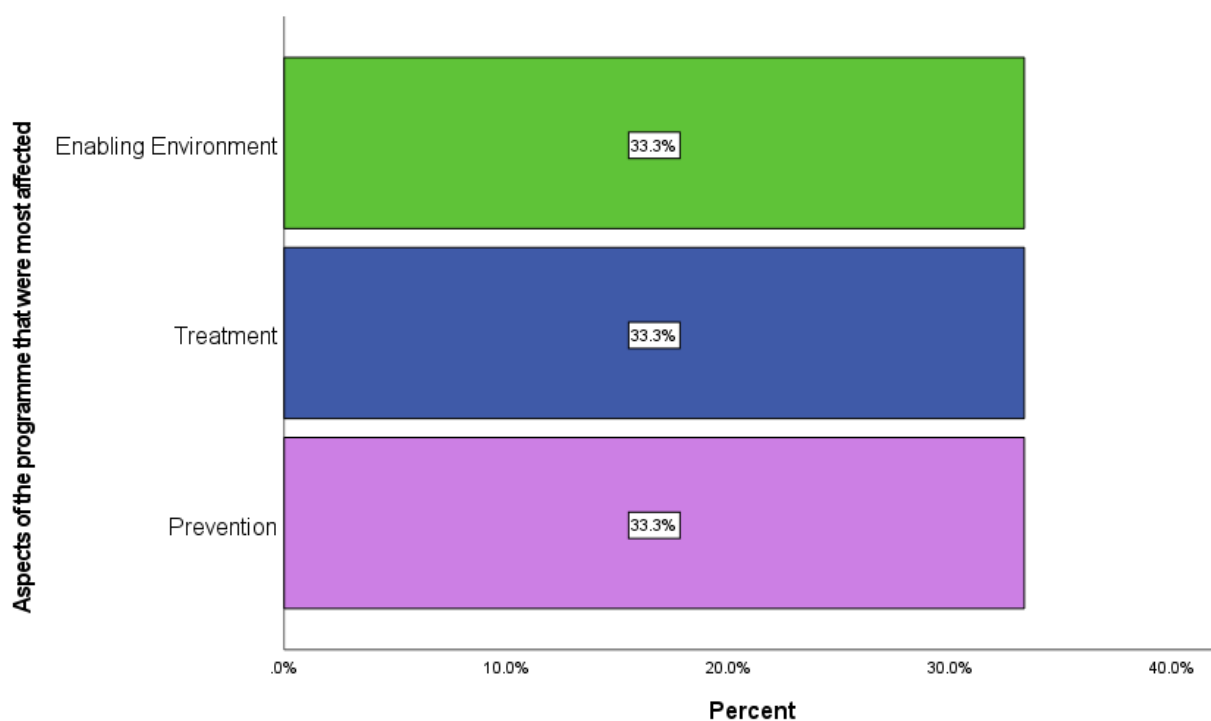


Figure 16: Aspects of the service delivery that were most affected due to the pandemic.

The COVID-19 pandemic has disrupted the service delivery of many Civil Society Organisations (CSOs) across Jamaica. Table 13 presents the rating of the officers of the CSOs on selected issues. The officers of the CSOs believed that curfew and travel restrictions and reduced opening hours significantly hampered their service delivery. However, they were uncertain whether working from home, comorbidities, or clients' geographic place of abode adversely influence their service delivery.

Table 13
The officers of the CSOs rating the level of disruption of your service delivery caused by selected issues.

Details	1	2	3	4	5
Curfew, restricted travel	0.0	0.0	0.0	66.7	33.3
Mask wearing	33.3	33.3	33.4	0.0	0.0
Reduced opening hours	0.0	0.0	0.0	100.0	0.0
Social distancing	0.0	33.3	33.3	33.4	0.0
Vaccination status	0.0	33.3	33.3	33.4	0.0
Working from home	0.0	0.0	100.0	0.0	0.0
Clients' geographic location	0.0	0.0	66.7	33.3	0.0
Comorbidities	0.0	0.0	66.7	33.3	0.0
Increased GBV	0.0	0.0	50.0	50.0	0.0

1-Very low, 2 – low, 3-moderate 4 – high, and 5 – very high

The officers of the CSOs mostly indicated that the grant funding was adequate to support their programme needs during the pandemic (67%, n=4). Sixty-seven per cent of the officers of the CSOs indicated that the grant funding was adequate to the programme's need for the pandemic, this means that some CSOs were faced with the challenge of inadequate funding to address the needs of their clients during the pandemic.

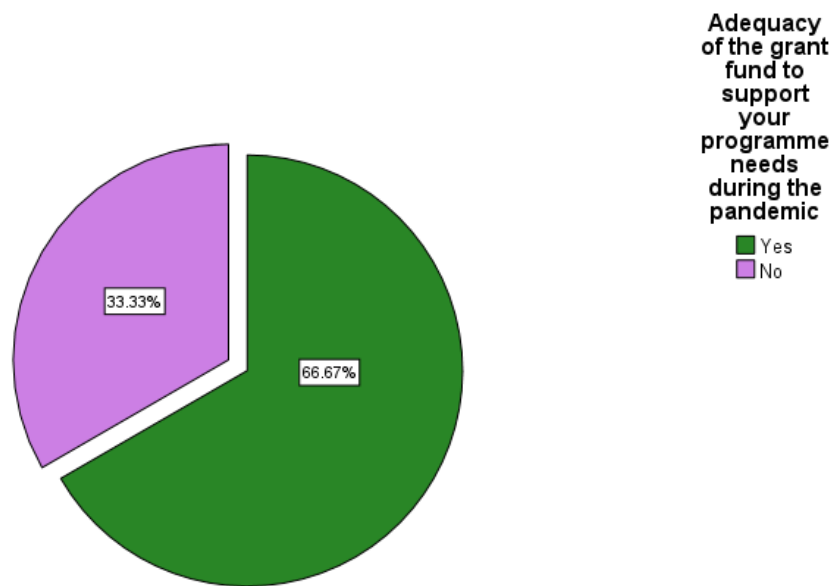


Figure 17: Adequacy of the grant funding to support the programme needs during the pandemic

The officers of the CSOs indicated that infected staffers (67%) adversely influenced their service delivery. Furthermore, they were uncertain as to whether disruption in service delivery was caused by the mobility of staff (67%), staff absenteeism (67%), and shortage of critical supplies (67%), increased community engagement (100%), supporting income generating projects (100%), and procurement (67%).

Table 14

The officers of the CSOs rating the level of disruption to your service delivery caused by each of the fallouts from the COVID-19 pandemic.

Details	1	2	3	4	5
Infected staff (with COVID-19)	0.0	0.0	0.0	66.7	33.3
Absence of staff	0/0	0.0	66.7	33.3	0.0
Mobility of staff	0.0	0.0	66.7	33.3	0.0
Vaccination status	0.0	33.3	33.3	33.4	0.0
Shortage of critical supplies caused by COVID-19	0.0	33.3	66.7	0.0	0.0
Providing shelter	0.0	100.0	0.0	0.0	0.0
Increased community engagement	0.0	0.0	100.0	0.0	0.0
Supporting income generation	0.0	0.0	100.0	0.0	0.0
Provide back-to-school support for clients	0.0	66.7	33.3	0.0	0.0
Procurement of Personal Protective Equipment	0.0	33.3	66.7	0.0	0.0

1-Very low, 2 – low, 3-moderate 4 – high, and 5 – very high

During the focus group session, there was a shared consensus among members of the CSOs that the government protocols (lockdown, no movement for non-essential workers), the lack of information and misinformation, initially brought the services to clients to a snail's pace and for some, it was a halt. The unpreparedness of the CSOs resulted in lowered service delivery, and this was noted by the clients. With time the CSOs implemented appropriate and suitable measures that allow for the provision of the necessary services.

In an interview, the Project Manager from the Ministry of Health and Wellness (MoHW), Dr Nicola Skyers admitted that no one was prepared for the COVID-19 pandemic and concurred with the members of the CSOs that the fund was allocated during the mid-to-latter part of the pandemic; but that this should not have substantially hampered the service delivery of the organizations. From the interview with Dr Skyers and the focus group session with the members of the CSOs, there is a consensus that there would be some period of a break in service delivery; but the CSOs seem to have been handicapped for too long a period. The unexpectedness of the pandemic and not having all the scientific information at the beginning undoubtedly created a gulf between response and execution.

Dr Skyers opined, "There seemed to have been challenges in terms of the activities as per the approved work plan. In fact, some of the entities basically started implementing the activities in the second year which started in October 2021. So, lots of entities haven't implemented activities as per the year-one portion of the grant. I know that for some they had indicated that several of the activities were under budgeted". There seems to have been a glitch in communication between the MoHW and the CSOs in the execution of the work plan during the pandemic, and Dr Skyers postulated, "A consultant would have been recruited to work with the entities to develop their work plans, so these would then have been collated and submitted to the ministry for review to ensure

that the activities match as well as we are within budget. So, this is a document that would have been submitted by the CSOs for final approval. I'm not sure where there were some communication challenges, but some persons had in fact indicated that they weren't familiar with some of these activities.” The slow response of the CSOs to the pandemic hampered not only the service delivery but intensified the fear of the clients, not knowing what was happening during the early stage of the pandemic. The lack of information on the virus, the ill health of some of the service delivers, and the low response of the CSOs interfered with the experiences of the clients during the pandemic. The clients were able to identify the gaps in service delivery and attributed this to the COVID-19 pandemic, suggesting that the pandemic significantly retarded service delivery.

Mr Cruickshank concurred with the CSOs and Dr Skyers that no entity was completely or partly prepared for the COVID-19 pandemic, and that people were responding as information developed. He chided the government for initially excluding the CSOs from the general discussion and planning responses to the pandemic and noted that still hampered the provision of services at the beginning of the pandemic. To support his perspective, Mr Cruickshank provided the example of the impulsive response of the government to the first reported and confirmed case of COVID-19. He articulated that the first confirmed case was in Bull Bay, St. Andrew, and the government order a shutdown of the community. This meant that some of the clients of the CSOs were unable to access services including medical supplies. On the onset, CSOs’ representatives were unable to visit clients within this community, and so this interfered with the outreach activities of the CSOs.

The interferences were sometimes outside of the scope of the CSOs to include the medication supplies and the medical distribution centres. Mr Cruickshank opined that medical supplies were restricted because the Global Supply Chain was unable to fulfil orders as obtained before the pandemic. Another aspect of interference was restriction based on the number of people

who could be served at clinics, hospitals, and DrugServ. Those challenges created a bottleneck at medical distribution sites as well as the early human mobility restrictions implemented by the government interfered with the service delivery to the clients. Mr Cruickshank noted that the number of unknowns that emerged due to the COVID-19 pandemic made it increasingly difficult for the CSOs to completely respond and address the myriad of challenges of the clients. Despite those issues, the CSOS were able to recalibrate their operations and new measures were implemented to address the realities of the clients.

How was the COVID-19 response of Global Fund used to effectively assist in overcoming the disruption of the critical services provided by the CSOs?

Table 15 presents the officers of the CSOs' responses to rating how did the grant from the Global Fund assist them in overcoming the disruption to your critical services. The findings revealed that the grant funding was mostly inadequate to overcome the disruption of the services provided by the agencies. In fact, in only two cases were the funds able to overcome some of the disruptions caused by the pandemic (i.e., operating the clinics and providing food).

Table 15

The officers of the CSOs rating how did the grant from the Global Fund assist you in overcoming the disruption to your critical services.

Details	1	2	3	4	5
Operation of clinics	0.0	0.0	50.0	0.0	50.0
Field visits	0/0	33.3	33.3	0.0	33.4
Providing monetary support	0.0	0.0	66.7	33.3	0.0
Providing food support	0.0	0.0	33.3	33.3	33.4
Providing medication	0.0	50.0	50.0	0.0	0.0

1-Very low, 2 – low, 3-moderate 4 – high, and 5 – very high

The officers of the CSOs were asked, "were other resources (i.e., financial, etc.) used to bolster services during the COVID-19 pandemic?" their response is shown in Figure 18. All the officers indicated that other resources were employed to address some of the challenges caused by the pandemic.

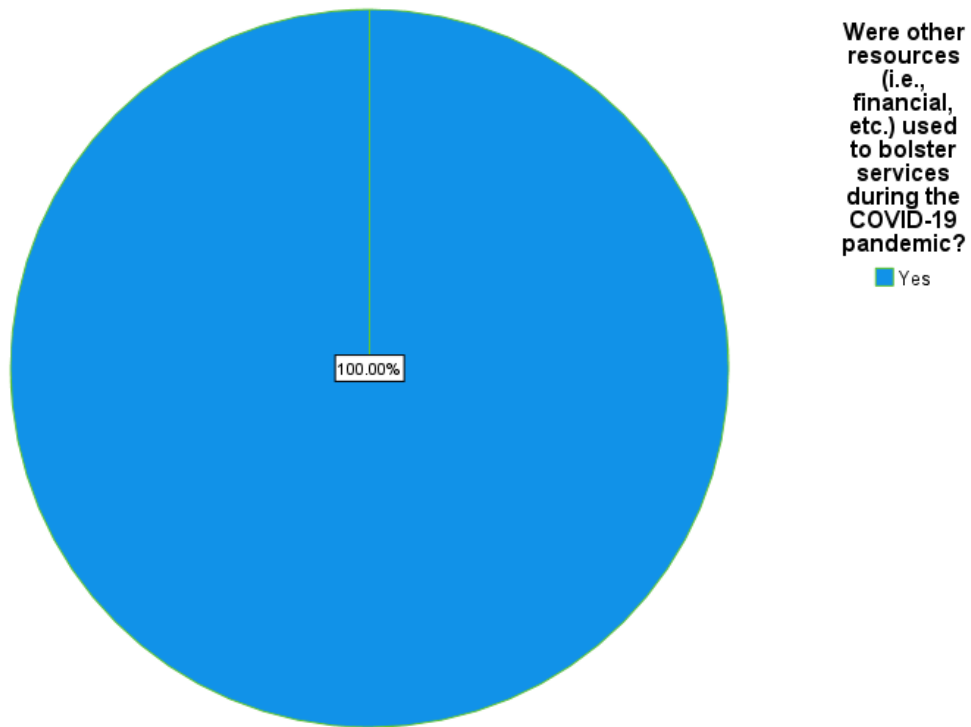


Figure 18: Other resources used to bolster services during the pandemic.

To understand the extent of the shortfall of the Global Fund COVID-19 Response Grant to address the challenges caused by the pandemic, Table 16 provides some insight into this issue. Table 16 revealed that in some cases 81-100% of the grant funding was spent during the pandemic, and at least 41-60% of the grant budget was used to address challenges caused by the pandemic. In addition, the officers indicated that resources from other budgets were used to bolster services during the pandemic (1-20%).

Table 16
The Officers' perspective on selected issues

Details	1	2	3	4	5	6
What per cent of other budgets were used to bolster services during the COVID-19 pandemic?	0.0	66.7	33.3	0.0	0.0	0.0
What per cent of clients were used to serve during the COVID-19 pandemic?	0.0	100.0	0.0	0.0	0.0	0.0
What per cent of your clients have you served during the COVID-19 pandemic?	0.0	0.0	0.0	33.3	66.7	0.0
What per cent of the grant budget was spent?	0.0	0.0	0.0	33.3	33.3	33.4

Noted: 1=0%; 2=1-20%; 3=21%- 40%; 4=41%-60%; 5=61%-80%, and 6=81%-100%

The focus group provides a critical set of information on the use of the Global Fund. The members of the CSOs indicated that most of the funds were not expended during the pandemic because the money was received towards the end of the pandemic. The information provided in Table 16 supports the reallocation of funds from other projects towards COVID-19; but there is no evidence that 70% of the Global Fund COVID-19 Response Grant was used during the pandemic.

The officers of the CSOs have admitted that they were very highly effective in employing selected issues during the COVID-19 pandemic (Table 17). However, they indicated that at least high effectiveness in providing social and material support (100%), supporting infrastructure and sustainability of service delivery (67%), collaboration, communication and capacity building (67%), and access to justice and legal redress (34%).

Table 17

The officers of the CSOs rating the effectiveness of their service delivery during the COVID-19 pandemic

Details	1	2	3	4	5
Collaboration, Communication and Capacity Building	0.0	0.0	33.3	33.3	33.3
Supporting infrastructure and sustainability of service delivery	0.0	0.0	33.3	66.7	0.0
Providing Social and material support	0.0	0.0	0.0	100.0	0.0
Access to Justice and Legal Redress	0.0	33.3	33.3	33.4	0.0

Note: 1-Very low, 2 – low, 3-moderate 4 – high, and 5 – very high

Figure 19 depicts the perspective of the officers of the CSOs' response to the per cent of the fulfilment of their service delivery during the pandemic. The findings revealed that none of the CSOs fulfilled 100% of their service delivery during the COVID-19 pandemic. However, they indicated that between 44 and 88% of their targeted service deliverables were accomplished during the pandemic. The CSOs' service deliverables during the pandemic were 33% for 45-65%, 85%, and 87%.

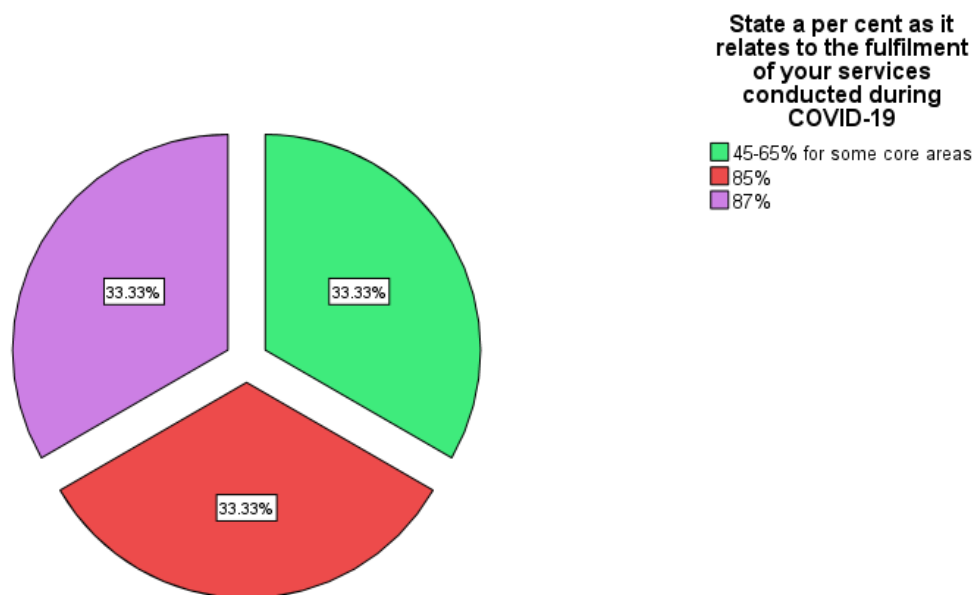


Figure 19: CSOs' response to the fulfilment of service deliverables during the pandemic

The officers of the CSOs indicated that more could have been done with more resources. The officers believed that service deliverables could have been greater with more resources (Figure 20).

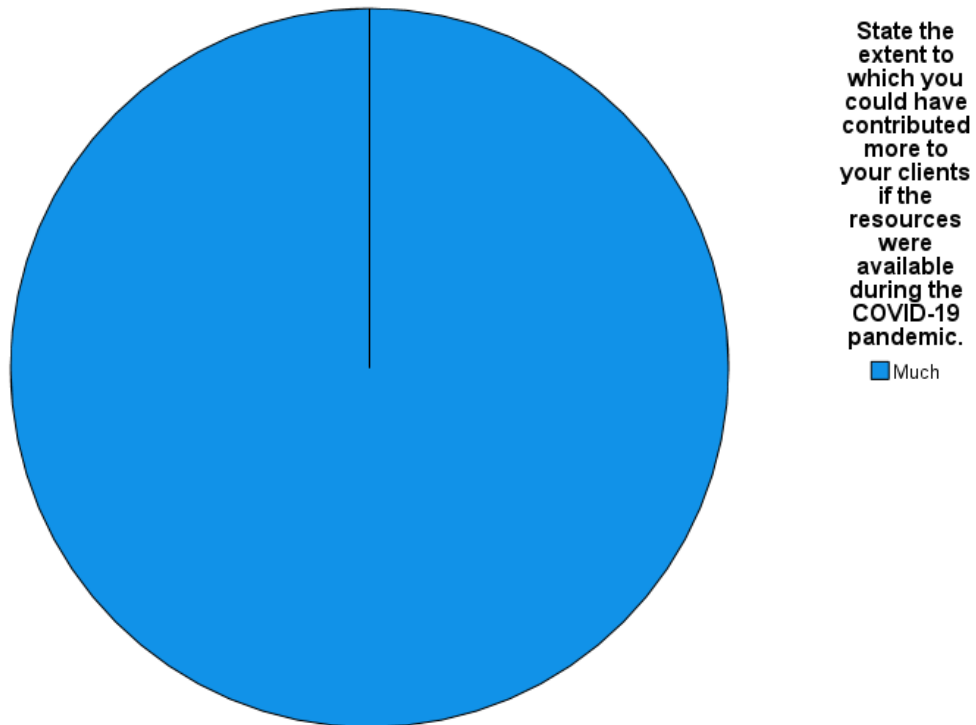


Figure 20: CSOs' response to the extent that more could be done with more resources.

Some people in the vulnerable population were not able to access services from the CSO because of the implemented government regulatory restrictions. Although the government regulatory restrictions were implemented to protect the wider population from the pandemic, they retard critical services to vulnerable communities. The officers of the CSOs indicated that they were hampered from executing their duties because of the curfews, lockdown, vaccination hesitancy, fear of contracting the virus, and travel challenges (Figure 21).

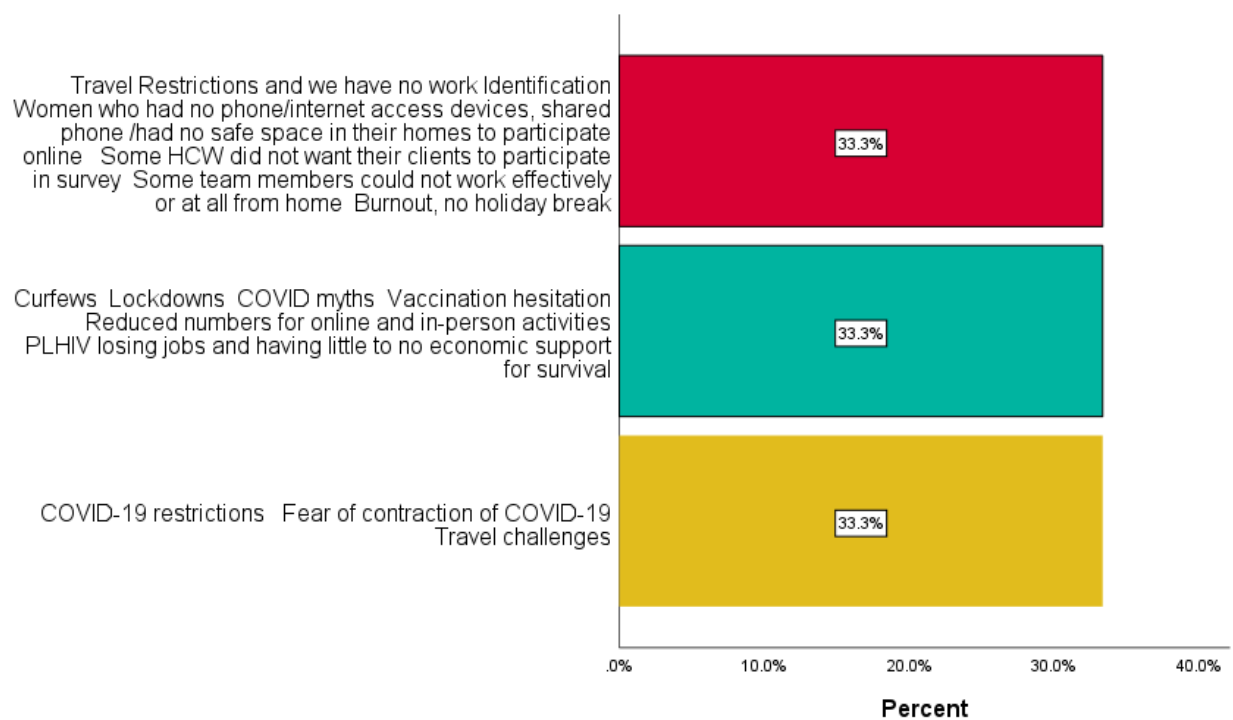


Figure 21: Challenges encountered while conducting activities during the pandemic.

The COVID-19 pandemic brought with it a plethora of challenges, and the Global Fund was used to alleviate some of those issues. The CSOs were able to provide various support for the vulnerable population in Jamaica. Table 18 presents a listing of services rendered to the vulnerable population during the pandemic. The officers of the CSOs were able to provide financial support, medication supplies, educational training, and counselling, among others services during the pandemic.

Table 18

Service delivered during the pandemic

Details
Counselling service and Art therapy
Care packages (food supplies and toiletries)
Sanitization packages
Medication
Virtual sessions (skills training)
Virtual educational sessions including the impact of the pandemic on HIV
PPEs
Online capacity building
Access to justice and redress for discrimination and GBV
Emotional support
Need Assessment Surveys
Financial support (including phone credit)

Table 19 presents the views of the clients and the officers of the CSO on ranking the types of support received and delivered, respectively. There is a convergence between the services delivered by the CSOs and services received by the clients of the CSO. Both parties agreed with the service deliverables and receivables during the COVID-19 pandemic.

Table 19

Ranking support based on the views of the Clients and Officers of the CSOs

Details	Clients	Officers of the CSOs
Care Packages	1st	1st
Financial support (including site visits)	2nd	2nd
Medical support	3rd	3rd
Counseling	4th	4th

The clients and the officers of the CSOs were asked to rate the effectiveness of their service delivery and the views are presented in a single Table (Table 20). The views expressed by both parties were similar, with there being a single difference in the rating of providing social and material support. The Officers of the CSO rated providing social and material support to the clients as high (100%); but the clients rated the same issue at 33.3% and very high at 33.4%. On the issue relating to access to justice and legal redress, the clients were mostly indecisive about this phenomenon, which was not the case among the Officers of the CSOs. The Officers of the CSO were indecisive on the matter of access to justice and legal redress during the pandemic; but they were less indecisive than the clients.

Table 20

The Clients and Officers of the CSOs rating the effectiveness of their service delivery during the COVID-19 pandemic.

Details	Clients					Officers of the CSOs				
	1	2	3	4	5	1	2	3	4	5
	%	%	%	%	%	%	%	%	%	%
Collaboration, Communication and Capacity Building	0/0	0.0	0.0	66.7	33.3	0.0	0.0	33.3	33.3	33.3
Supporting infrastructure and sustainability of service delivery	0.0	0.0	33.3	66.7	0.0	0.0	0.0	33.3	66.7	0.0
Providing Social and material support	0.0	0.0	33.3	33.3	33.4	0.0	0.0	0.0	100.0	0.0
Access to Justice and Legal Redress	0.0	0.0	66.7	33.3	0.0	0.0	33.3	33.3	33.4	0.0

Note: 1-Very low, 2 – low, 3-moderate 4 – high, and 5– very high

Although the COVID-19 Response grant from the Global Fund (Fund) arrived in the midst of the COVID-19 pandemic, it played a major role in providing critical care packages, phone credit, financial support, medical and educational expenses, and other support for the clients of the CSOs in Jamaica. From the focus group session, before receiving the fund, money was reallocated from other programmes and some bureaucracies in the distribution process hampered a more extensive coverage of needs for the clients. Ms. McKenzie (pseudo name) opined that the *“bottlenecks caused by bureaucracy slowed the flow of funds to CSOs”* and this fact lowered responsiveness and full coverage of the CSOs. It seems like the *bottlenecks* to which the members of the CSOs speak were the late arrival of the funds and Dr Skyers concurred with this truth. However, she argued that they (CSOs) were instructed to divert funds to respond to the pandemic and that this would be reimbursed at a later date, which was to be from the Global Fund

Despite the timing of the allocation of the Global Fund COVID-19 response grant, the members of the CSOs lauded the importance of the Fund to meeting the needs of their clients. The clients were equally and simultaneously concurring with the CSOs that the resources were allocated during the pandemic and that they were, for the most part, effective in meeting a large per cent of their needs. Dr Skyers opined:

They were given additional resources to build out their ICT infrastructure consumables and supplies and the phone cards, Internet packages etc. to support them in that kind of transition because face- to-face interactions were therefore limited particularly in year 1. So those things would have been taken into consideration. We knew that it definitely would have impacted, but I wouldn't say that is the major reason for any shortfall that they would have because they too would have transitioned to online services. In more recent times each organisation has been required to develop a risk register and that is now being

integrated in the work that they do, and they do provide reports in regards to the risk register.

Dr Skyers' perspective provides some context for the utilization of social media, disbursement of sums for phone cards, and other methods during the pandemic to supply the needs of the clients. The views of the clients, and the benefactors, provide a comprehensive understanding of the reach/coverage and effective of engagement of the CSOs. Dr Skyers said *inter alia* "the indicators are designed to ensure people were receiving their test et cetera. But the indicators don't necessarily take into consideration the qualitative component of the work".

This study provides qualitative responses for the clients on how they were provided for during the pandemic. The clients indicated that they were provided for during the pandemic, except for a brief period of care.

On the other hand, Dr Skyers remarked, "It's very quantitative in terms of how the measurement is done --another person needed to get food packages and that sort of thing. But nobody really sits down and counts how many persons are able to stay on their medication because of the support they are getting. So, we do recognize the absence of the qualitative component. It's quite important, but based on how the global fund measures, like industries, it's purely numbers." This study provides a quantitative aspect of the clients' views on benefits and operations during the pandemic, and majority of them indicated a high degree of satisfaction with the services they received during the period.

Despite the high level of satisfaction on service delivery in meeting the needs of the clients, Dr Skyers offered that there were stated objectives embodied in the Work Plans of the CSOs for assessing effectiveness of the COVID-19 response grant. The rationale for the identification of a

Work Plan was for a quantitative assessment of the grant funding. So, it was important to develop a plan.

An elite interview with Mr Ivan Cruickshank of the Country Coordinating Mechanism in Jamaica (CCM) provided greater insight on some challenges encountered by the CSOs. He explained that although the work plan was developed by a consultant, it was in consultation with the CSOs. Mr Cruickshank noted that there were some issues with aspects of the work that would have accounted for some delays in execution. The executive director of JASL, Mrs. Kandasi Watson-Levermore, indicated that the Ministry of Health and Wellness was asked to provide a coordinator who would be responsible for the coordination of all activities relating to the COVID-19 response to clients, but this was never provided or an allocation made in the budget for this expense. Mr Cruickshank explained that the non-provision of a coordinator hampered aspects of the work plan as JASL had to stretch the already meagre staff and assign someone to act as a coordinator. Clearly, this thwarted scheduled and timely execution of aspects of the work plan, which was somewhat outside of the CSOs' control.

The purpose of the work plan was to reach vulnerable populations during the pandemic. Dr Skyers aptly summarized the importance of the work plan, "There are no specific targets for the COVID-19 grant, we would have had targets under the work plan in terms of reaching the various populations. So, there was nothing special. I mean it's an emergency. What we do routinely is that we have quarterly reports; and depending on the organization, we may have monthly meetings with them to review the implementation of the activities under the grant, not just limited to COVID-19, but also to the main grant that is being implemented. So, that is how we monitor the implementation of all our grants."

Based on the quantitative aspect of this study, the reach of the response initiatives by the CSOs was extensive, and the coverage to meet the needs of the clients was over 70%. The coverage per cent indicates that there were underserved people, and this may be due to the increased expectations from the worsened socioeconomic state of the clients.

How was the preparedness of the CSOs for a pandemic prior to the COVID-19 pandemic?

The COVID-19 pandemic was a surprise to people and entities in Jamaica. CSOs were occupied with a plethora of existing challenges prior to the pandemic, and this was exacerbated by the virus. Before COVID-19, the CSOs in Jamaica were actively engaged in addressing the concerns and needs of their clients, and this was done with limited existing resources. During the focus group session, all the CSOs reported that they were not prepared for the COVID-19 pandemic and that they had to divert resources from other programmes/projects in order to immediately respond to the pandemic. Dr Skyers from the Ministry of Health and Wellness (MoHW) echoed the sentiments of the members of the CSOs that no one was prepared for the pandemic.

The CSOs in attendance indicated that they had a strategic disaster protocol; but it did not include a pandemic. They postulated that the COVID-19 was different from the issues contained in existing disaster protocols and they had to improvise to deal with the new realities. One CSO indicated that he experienced severe challenges to access information on the pandemic and initially he was excluded from policy planning meetings with the government and/or the Ministry of Health and Wellness. The perspective of Amos (pseudo name) was also echoed by all the agents present at the session. During the early stage of the pandemic, the lack of information and misinformation on the virus coupled with the psychology of fear for the virus as well as governmental restrictions

made it different from challenges faced in the past. All those issues caused a break in service, and they intensified the expectations of the clients.

Dr Skyers opined: “To provide resources to help CSOs to mitigate the challenges caused by the pandemic in delivering benefits to beneficiaries and that embedded in the purpose of the Global Fund was meeting the needs of the served clients. The fact is the expectations of the clients and the reality of their situation demanded more resources, which is somewhat understandable according to Dr. Skyers. The reality is the CSOs had desired a system of ensuring that all the clients are served and that they would not roam all the agencies for the same benefits.

The Global Fund’s assistance came towards the end of the heightened period of the pandemic. The much-needed fund replaced the other diverted funds. Many of the employees of the CSOs were ill, and there were no funds to acquire other skilled workers. The issue of human resources was crippled during the pandemic, this was unplanned and so innovative measures had to be employed to supply the needs of the clients. The CSOs reiterated that not having reserved funds, an established pandemic protocol, and money for administrative expenses crippled the initial response of the agencies. Phone credits and social media platforms were employed to engage the clients, and this meant managing a new reality. The CSOs opined that many lessons have been learnt from the pandemic; but that there is an urgent need for a robust pandemic protocol that includes them as critical elements in a first response protocol.

Discussion and Conclusion

The value of the utilization of the Global Fund's COVID-19 Response Grant must be assessed by way of the qualitative assistance offered to vulnerable groups in Jamaica during the pandemic. As mentioned earlier in this study, some of the realities of the COVID-19 pandemic are domestic violence, sexual violence, and other maladaptive behaviours that emerged because of longer periods at home. The associated challenges of the pandemic were reflected in studies by Bourne et al which found that 38.2% of Jamaicans indicated they were practicing safe sex [20], 7.3% increase in Jamaicans having more sexual partners during COVID-19 and an increase in sexual engagement [21]. In addition, there were many cases of domestic violence (32% of women during the pandemic) [22], 60.0% were physical, 31.1% were emotional abuse, and 8.9% were sexual abuse [22]. The quantitative issues discovered by Bourne et al. could be worrying if we take into consideration the impact on some of the people from the vulnerable groups in Jamaica, particularly those currently who are HIV-positive and/or AIDS patients.

COVID-19 was declared a pandemic in 2020 by the World Health Organization [80]. It has changed the geopolitical and social landscape of our contemporary society. The pandemic has intensified the economic burden of the vulnerable population, and this has increased among those with comorbidities. Assistance from the Global Fund was timely, and it provided needed alleviation for the vulnerable population. The vulnerable population in Jamaica experienced many socioeconomic challenges during the COVID-19 pandemic, and CSOs were able to navigate many of those issues and ultimately provide for their clients.

The last pandemic before COVID-19 was in 2009, H1N1(pdm09 virus), and before this were H3N2 virus (9168), H2N2 (1957-1958), and H1N1 (1918) [81-84], with the most severe in recent history being H1N1 virus in 1918-1919 [83]. This means that the last devastating pandemic

in recent history was over a century (in 1918-1919) ago, and only a few Jamaicans from that era currently exist. On March 10, 2020, the first COVID-19 case was confirmed in Bull Bay, St. Andrew, Jamaica, and this created a new reality. The virus then spread across the fourteen parishes in Jamaica, and so were the numbers of mortality [85-87]. The lack of information on the virus at the onset explains why some Jamaicans were intolerant to those who appeared to be carriers of the virus, as evidenced by a man being beaten and thrown off a bus after sneezing [88].

COVID-19 was a global communicable disease, and Jamaicans were not only unprepared as they were indifferent to those with or perceived to have virus. Members of the CSOs were equally apprehensive about COVID-19; but they had to respond despite the lack of information and their fears of the unknown. Hence, the CSOs had to respond to their clients despite the COVID-19 pandemic, the unknown about the virus, ill-health among its members, and protocol restrictions by the Jamaican government. During this time, the clients were in fear of their survivability simply because the restrictions affected their well-being by reducing access to medications and other supplies. After an initial delay, the CSOs were able to meet many of the clients' needs during the pandemic.

Although the CSOs were initially hampered by the governmental protocols (i.e., lockdowns, no human movement, early closing hours for businesses, and exclusion from national resource planning committees and practices) and internal challenges such as ill workers and not having a national COVID-19 response coordinator), they were able to provide needed services to the at-risk population. The current findings revealed that most of the sampled clients indicated that they received services provided by the CSOs (91.03%), and that they were moderately satisfied with the services received (63.7% of the clients indicated that they were at least 75% satisfied with the service delivery of the CSOs during the pandemic). Clearly, there is evidence that the at-risk

population substantially benefited from the contribution, efforts, and work of the CSOs during the pandemic. Despite the efforts of the CSOs, there were some dissatisfied clients (i.e., 17.1% were at most 20% satisfied with the service delivery during the pandemic) and this may be due to a myriad of issues.

The CSOs explained that they coordinated among themselves, and protocols were established for the distribution of benefits to needed clients. All the CSOs indicated that some of the clients wanted to access resources from various agencies, and they were prevented by the established protocols. Mr Cruickshank referred to this desire by the clients as the ‘Anansi culture’ in Jamaica. The ‘Anansi culture’ is one in which people seek to trick others, schemed for resources, and by enlarge use whatever means to acquire things. Like Mr Cruickshank, the CSOs believed that by retarding the ‘Anansi culture’ of some of the clients, it is understandable why some would have been dissatisfied about service delivery. Outside of the ‘Anansi culture’, the restrictions caused by the government protocols would have accounted for an aspect of the dissatisfaction as some clients would not have been serviced as was previously the case before the pandemic. The current findings revealed that there was a 20.1% decline in excellence of self-reported general health status of clients during the pandemic, which accounts for some degree of the dissatisfaction. Although this may not be as a result of the service delivery or lack thereof by the CSOs, it would have contributed to the clients’ level of dissatisfaction.

The response of the CSOs to the service delivery during the COVID-19 pandemic was different and so clients were provided with phone credit, online social support mechanisms, care packages, and online counseling services. Those approaches were effective in addressing many of the new realities experienced by clients during the pandemic. The current findings revealed that at least 67% of the clients gave a high rating for the effectiveness of social and material support;

supporting infrastructure and sustainable services; and collaboration, communication and capacity building during the pandemic. The CSOs were able to effectively meet the needs of the clients because of a pre-designed work plan.

The pre-designed work plan provided a framework that was used to respond to the COVID-19 pandemic. The matter of not having a coordinator to execute and monitor the work plan or the absence of budgetary allocation for such a service contributed to the delay in executing the work plan. The delay in service was noted by some of the clients, although there were CSOs who indicated that there was no gap in service delivery during the pandemic. Nevertheless, all the CSOs lamented their exclusion from the planning, monitoring, and coordination efforts of the government to respond to the pandemic, and that this affected their service delivery during the initial period. Mr. Cruickshank observed that the response of the Government to the pandemic initially excluded the social and psychological aspects of people. The perspective of Mr. Cruickshank is supported by the current findings as there was a decline in the excellent self-reported general health status of clients during the pandemic. One can extrapolate from the current findings that psychosocial response of the CSOs cushioned the negative effects of the pandemic, and without their contributions the effect of the pandemic would have been more devastating on the clients' well-being. The COVID-19 grant from the Global Fund provided significant assistance and relief for the clients, without which many would have extensively suffered from the psychosocial pressure caused by a medical pandemic.

As Mr. Cruickshank opined, “there were so many unknowns about COVID-19, and this made it difficult to completely address”. COVID-19 was declared a pandemic by the World Health Organization in 2020 [80] and since that time we continued to learn about the virus. Many lessons have been learnt from the pandemic, which has taught us to prepare for similar hazards in the

future. The current study recognizes that high unpreparedness of the CSOs warrant a pandemic emergency preparedness and business continuity management framework. This framework will be provided here, and hopefully it will not be needed any time soon; but the CSOs in Jamaica will be better prepared peradventure a pandemic or hazard of a similar scale occurs in the future.

In concluding, the COVID-19 response that was made possible through the Global Fund has aided in cushioning the negative effects of the pandemic on clients served by Jamaican CSOs.

The COVID-19 pandemic was unplanned for, and many people regrettably lost their lives during the period; but many lessons were simultaneously learnt that allows for the development of a response and recovery framework. This is addressed in part two of this study.

Recommendations

The gaps identified by this study allows for the following recommendations:

1. Policy framework:

- i. There is an urgent need to develop a national policy response-framework that includes the CSOs as key stakeholders
- ii. The Government needs to include CSOs as a part of pandemic response as they are actively engaged in the communities and so will be able to quickly and timely meet the needs of vulnerable populations
- iii. The CSOs need to design a response framework for public health outbreaks and other emergencies
- iv. The Government needs to include a national response-coordinator and island-wide response teams for public health outbreaks to liaise with CSOs

2. Research:

- i. Studies are needed to ascertain the nature, scope, and challenges experienced by the vulnerable population and sub-populations during the COVID-19 pandemic. These will provide insight on shortfalls and how to address them peradventure a pandemic occurs in the future
- ii. A research databank is needed to collect data on the psychosocial issues experienced during the COVID-19 pandemic and this should be used to develop policy formulations and intervention strategies.

References

- [1] Centres for Disease Control and Prevention (CDC). First Report of AIDS. Mortality and Morbidity Weekly Report, 2001; 50(21):429-456.
- [2] Washington State Department of Health (WSDH). Washington State HIV Surveillance Report. Washington: WSDH; 2020. <https://doh.wa.gov/sites/default/files/legacy/Documents/Pubs//150-030-WAHIVSurveillanceReport2020.pdf>
- [3] Pierce County AIDS Foundation (PCAF). PCAF history. Washington: PCAF; 2022. <http://www.pcaf-wa.org/about/pcaf-history/>
- [4] U.S. Department of Health & Human Service. A Timeline of HIV and AIDS. Washington DC: U.S. Department of Health & Human Service; 2022. <https://www.hiv.gov/hiv-basics/overview/history/hiv-and-aids-timeline>
- [5] Ministry of Health & Wellness (MoH). HIV in Jamaica: Historical timeline. Kingston: MoH; 2020. <https://hstu.moh.gov.jm/information-on-hiv-sti-tb/hiv-in-jamaica-historical-timeline/#:~:text=HIV%2FAIDS%20became%20an%20issue,with%20the%20parishes%20of%20St.>
- [6] Figueroa JP, Duncan JP, Bailey A, Skyers N. The HIV epidemic in Jamaica: a need to strengthen the National HIV Program. Rev Panam Salud Publica. 2020 Nov 20;44:e157. doi: 10.26633/RPSP.2020.157. PMID: 33245295; PMCID: PMC7679044.
- [7] Olukoga A. Epidemiologic trends of HIV/AIDS/ Rev Panam Salud Publica/Pan Am J Public Health 2004; 15(5): 358-363.
- [8] UNAIDS. Global AIDS response progress report. https://www.unaids.org/sites/default/files/country/documents/JAM_narrative_report_2014.pdf
- [9] Ministry of Health (MoH). HIV epidemiological profile 2015: Facts & Figures. Kingston: MoH; 2015. <https://moh.gov.jm/wp-content/uploads/2017/05/2015-Epi-Update-Revised-Final.pdf>
- [10] Figueroa JP, Duncan JP, Bailey A, Skyers N. The HIV epidemic in Jamaica: a need to strengthen the National HIV Program. Rev Panam Salud Publica. 2020;44:e157. <https://doi.org/10.26633/RPSP.2020.157>
- [11] Angus G. National HIV/AIDS response project receives \$1 billion. Kingston: Jamaica Information Service (JIS);2022. <https://jis.gov.jm/national-hiv-aids-response-project-receives-1-billion/>.
- [12] UNAIDS. Country: Jamaica. Geneva; 2022. <https://www.unaids.org/en/regionscountries/countries/jamaica>
- [13] Ministry of Health. HIV Epidemiology Report for the Ministry of Health, Jamaica. Kinston: MoH; 2017. <https://www.moh.gov.jm/wp-content/uploads/2019/10/2017-HIV-Epi-Profile-Jamaica.pdf>.
- [14] Figueroa JP, Duncan J, Byfield L, Harvey K, Gebre Y, Hylton-Kong T, Hamer F, Williams E, Carrington D, Brathwaite AR. A comprehensive response to the HIV/AIDS epidemic in Jamaica: a review of the past 20 years. West Indian Med J. 2008 Dec; 57(6):562-76.

- [15] Bourne PA, Palmer A, Brady B, Swaby R, Malcolm D, Peterkin VMS, Fallah J, Campbell C, Foster C, McLean C, White M. An Evaluation of Domestic Violence against Jamaican Women during the Coronavirus disease 19 (COVID-19) Pandemic. *Global Journal of Transformation in Law, Human Rights and Social Justice*, 2021; 5(1): 18-40.
- [16] Bourne PA, Hudson-Davis A, Sharpe-Pryce C, Clarke J, Solan I, Rhule J, Francis C, Watson-Coleman O, Sharma A, Campbell-Smith J, Allen T. Does Marriage Explain Murders in a Society? In *What Way is Divorce a Public Health Concern?* *International Journal of Emergency Mental Health and Human Resilience*, 2014; 16(2): 84-92.
- [17] Bourne PA, Bent N, Blake R, Brackett C, Francis S, McInnes S, Fallah J, Campbell C, Foster C, McLean C, Muchee T, Biira B. The Disintegration of Jamaican Families: Using Divorce Data, 2001-2020. *International Journal of Transformation in English & Education*, 2022; 7(1), 17-36.
- [18] Bourne PA, Peterkin VMS, Brooks D, McLean C, Fallah J, Foster C, Biira B, Campbell C, Muchee T, Meikle AL. The Dilemma of Missing People in Jamaica: A Societal Disintegration of the Family Structure and a Gendered Phenomenon. *International Journal of Current Research in Education, Culture and Society*, 2022; 6(1), 24-49.
- [19] Bourne PA, Reid D-A, Plummer S, Lewis T, Ferguson J, Foster C, Burke M. The Influence of Alcohol Consumption on Sexual Promiscuity among Young Jamaican Males during the COVID-19 Pandemic. *International Journal of Recent Advances in Medical & Pharma Research*, 2020; 3(1): 23-35.
- [20] Bourne PA, Harvey T, Walters K, Scott S, Hibbert C, Burke M, Foster C. The Effects of the COVID-19 pandemic on the Sexual Behaviour, Sexual Health, and General Health Status of Jamaicans: Has the sexual behaviour of Jamaicans changed, or is it the same as before the COVID-19 pandemic? *Global Journal of Transformation in Pharmaceutical Formulation and Consumer Health*, 2021; 2(1): 1-15.
- [21] Bourne PA, Johnson S, Duffus A, Hart O, Walker S, Burke M, Foster C. The sexual behaviour of Jamaicans before and during the COVID-19 pandemic. *International Journal of Multidisciplinary Research and Growth Evaluation*, 2021; 2(3):390-398.
- [22] Bourne PA, Palmer A, Brady B, Swaby R, Malcolm D, Peterkin VMS, Fallah J, Campbell C, Foster C, McLean C, White M. An Evaluation of Domestic Violence against Jamaican Women during the Coronavirus disease 19 (COVID-19) Pandemic. *Global Journal of Transformation in Law, Human Rights and Social Justice*, 2021; 5(1), 18-40.
- [23] Jamaica Constabulary Force. Crime statistics. Kingston: JCF; 2022. <https://jcf.gov.jm/stats/>.
- [24] Mekouar D. Why homicide rates in the US spiked 30% during COVID pandemic. <https://www.voanews.com/a/why-homicide-rates-spiked-30-during-the-pandemic-/6420391.html>.
- [25] Meyer M, Hassafy A, Lewis G, Shrestha P, Haviland AM, Nagin DS. (2022) Changes in Crime Rates during the COVID-19 Pandemic, *Statistics and Public Policy*, 2022;9:1, 97109, DOI: [10.1080/2330443X.2022.2071369](https://doi.org/10.1080/2330443X.2022.2071369)
- [26] Boman JH 4th, Gallupe O. Has COVID-19 Changed Crime? Crime Rates in the United States during the Pandemic. *Am J Crim Justice*. 2020;45(4):537-545. DOI: 10.1007/s12103-020-09551-3. Epub 2020 Jul 8. PMID: 32837168; PMCID: PMC7340780
- [27] Fischer VJ, Bravo RG, Brunnet AE. et al. Sexual satisfaction and sexual behaviors during the COVID-19 pandemic: results from the International Sexual Health And REproductive (I-

- SHARE) health survey in Luxembourg. BMC Public Health 2022; 22, 1108. <https://doi.org/10.1186/s12889-022-13509-x>
- [28] Masoudi M, Maasoumi R, Bragazzi NL. Effects of the COVID-19 pandemic on sexual functioning and activity: a systematic review and meta-analysis. BMC Public Health, 2022; 22, 189. <https://doi.org/10.1186/s12889-021-12390>.
- [29] Eleuteri S, Terzitta G. Sexuality during the COVID-19 pandemic: The importance of Internet. Sexologies. 2021 January-March;30(1):e55–60. doi: 10.1016/j.sexol.2020.12.008. Epub 2021 Jan 2. PMCID: PMC7832719.
- [30] al Mamun F, Hosen I, Mamun MA. Sexual violence and rapes increment during the COVID-19 pandemic in Bangladesh. EClinicalMedicine, 2021; 34, 100817.
- [31] Mukhtar S, Mukhtar S. The Hidden Shadow Pandemic of Marital Rape During COVID-19 Pandemic Outbreak: A Critical Role of Women’s March for Awareness of Rape, Consent, and Sexual and Reproductive Rights. Journal of Psychosexual Health. 2022;4(2):80-94. doi:[10.1177/26318318221089415](https://doi.org/10.1177/26318318221089415).
- [32] Uwaegbute, K.I. & Unachukwu, D.C., 2022, ‘The upsurge of rape during the COVID-19 lockdown in Nigeria and its effects on survivors’, HTS Teologiese Studies/Theological Studies 78(3), a6996. <https://doi.org/10.4102/hts.v78i3.6996>.
- [33] Harvard Medical School Primary Care Review. A second, silent pandemic: sexual violence in the time of COVID-19. Boston: Harvard Medical School of Public Health; 2020. <https://info.primarycare.hms.harvard.edu/review/sexual-violence-and-covid>
- [34] Ogunlana MO, Nwosu IB, Fafolahan A, Ogunsola BF, Sodeke TM, Adegoke OM, Odunaiya NA, Govender P. Pattern of rape and femicide during COVID-19 lockdown: content and discourse analysis of digital media reports in Nigeria, Journal of Gender Studies, 2021. DOI: [10.1080/09589236.2021.2012134](https://doi.org/10.1080/09589236.2021.2012134)
- [35] Brown LB, Spinelli MA, Gandhi M. The interplay between HIV and COVID-19: summary of the data and responses to date. Curr Opin HIV AIDS. 2021 Jan;16(1):63-73. DOI: 10.1097/COH.0000000000000659. PMID: 33186229; PMCID: PMC7735216.
- [36] U.S. Department of Health & Human Service. COVID-19 and People with HIV. Washington DC: U.S. Department of Health & Human Service; 2022. <https://www.hiv.gov/hiv-basics/staying-in-hiv-care/other-related-health-issues/coronavirus-covid-19>.
- [37] USAIDS. COVID-19 and HIV. Geneva: USAIDS; 2022. <https://www.unaids.org/en/covid19>.
- [38] World Health Organization. HIV/AIDS services must remain crucial during the COVID-19 outbreak. Cité du Djoué: Regional Office for Africa; 2021. <https://www.afro.who.int/news/hivaids-services-must-remain-crucial-during-covid-19-outbreak>
- [39] Dawa N, Narain JP. COVID-19 & HIV/AIDS pandemics: Parallels & lessons. Indian J Med Res. 2021 Jan & Feb;153(1 & 2):214-218. DOI: 10.4103/ijmr.IJMR_2598_20. PMID: 33818479; PMCID: PMC8184063.
- [40] Some P, Eba PM. Lessons from HIV to Guide COVID-19 Responses in the Central African Republic. Health Hum Rights. 2020 Jun;22(1):371-374. PMID: 32669822; PMCID: PMC7348452.

- [41] The Global Fund. Response mechanism. 2022. <https://www.theglobalfund.org/en/covid-19/response-mechanism/>.
- [42] Jamaica AIDS Support for Life (JASL). Who we are: Our history. Kingston: JASL; 2021. <http://jasforlife.org/who-we-are/>
- [43] Centers for Disease Control and Prevention. HIV and COVID-19 basics. Atlanta: Centers for Disease Control and Prevention; 2022. https://www.cdc.gov/hiv/basics/covid-19.html?CDC_AA_refVal=https%3A%2F%2Fwww.cdc.gov%2Fcoronavirus%2F2019-ncov%2Fspecific-groups%2Fhiv.html
- [44] Pan American Health Organization. Coronavirus disease 2019 (COVID-19) and HIV: Key issues and actions. Washington DC: 2021. <https://www.paho.org/sites/default/files/2020-03/2020-cde-coronavirus-disease-hiv.pdf>
- [45] World Health Organization (WHO). Coronavirus disease (COVID-19) and people living with HIV. Washington DC: WHO; 2022.
- [46] Ministry of Health. National HIV/AIDS policy: Jamaica. Kingston: MoH; 2005. <https://moh.gov.jm/wp-content/uploads/2015/07/National-HIV.-AIDS-Policy.pdf>.
- [47] Navanti Group, UNICEF, UNAIDS. Assessing the impact of COVID-19 on people living with HIV, including pregnant women and children. Final Report; 2022. <https://www.unicef.org/moldova/media/7246/file/Assessing%20the%20impact%20of%20COVID-19%20on%20people%20living%20with%20HIV,%20including%20pregnant%20women%20and%20children.pdf>.
- [48] Jamaicans For Justice (JFJ). About us. Kingston: JFJ; 2022. <https://jamaicansforjustice.org/about-us/>
- [49] Jamaicans For Justice (JFJ). Universal periodic review of Jamaica. Kingston: JFJ; 2010. https://www.ohchr.org/sites/default/files/lib-docs/HRBodies/UPR/Documents/Session9/JM/JFJ_Jamaicansforjustice_Submission.pdf.
- [50] Jamaican Network of seropositive. Home. <https://www.myjnplus.org/>.
- [51] Jamaica Houses of Parliament. Submission - Jamaica Community of Positive Women (JCW+. Kingston: Jamaica Houses of Parliament; 2017. [https://japarliament.gov.jm/attachments/article/339/Submission%20-%20Jamaica%20Community%20of%20Positive%20Women%20\(JCW+\).pdf](https://japarliament.gov.jm/attachments/article/339/Submission%20-%20Jamaica%20Community%20of%20Positive%20Women%20(JCW+).pdf).
- [52] Jamaica Community of Positive Women, ICW Caribbean. WHIV Dialogue of Sexual Offences Act Recommendations. Kingston: JCPW; 2022. <https://jcwblogvocacy.wordpress.com/>.
- [53] Jamaica Community of Positive Women, ICW Caribbean. About us. Kingston: JCPW; 2022. <https://jcwblogvocacy.wordpress.com/about/>.
- [54] Ministry of Health. Jamaica Global AIDS Monitoring Report 2019. Kingston: MoH; 2019. https://www.unaids.org/sites/default/files/country/documents/JAM_2019_countryreport.pdf
- [55] TransWave JA. Annual Report 2020. TransWave Jamaica. https://issuu.com/transwaveja/docs/2020_annual_report_1.
- [56] UNAIDS. The power of transgender visibility in Jamaica. https://www.unaids.org/en/resources/presscentre/featurestories/2019/november/20191112_power-transgender-visibility-jamaica.
- [57] Devex. Equality for all foundation Jamaica Ltd. <http://www.devex.com/organizationss/equality-for-all-foundation-jamaica-ltd>.

- [58] Henry M. EFAF's 2021 Annual Report. Kingston; Equality for all Jamaica Foundation Ltd; 2022.
- [59] Eve for life. About Eve For Life . <http://www.eveforlife.org/page4.html#content1-2d>
- [60] The ASHE Company. About us. Kingston: The ASHE Company; 2018. <https://theashecompany.org/about-us/>.
- [61] Children First Agency. Who we are. St. Catherine: Children First Agency; 2022. <https://www.childrenfirstja.org/>.
- [62] Babbie E. The Practice of Social Research, 12th Edition. Wadsworth, a Division of Thomson Learning, Inc; 2010.
- [63] Polit DF. Data analysis and statistics for nursing research. Stamford: Appleton and Lange Publisher; 1996.
- [64] Rea LM, Parker RA. Designing and conducting survey research: A comprehensive guide, 4th ed. New Jersey: John Wiley & Sons, Inc; 2014.
- [65] Neuman WL. Social research methods: Qualitative and quantitative approaches, 7th ed. New York: Pearson/Allyn and Bacon; 2014.
- [66] Powell LA, Bourne P, Waller G. Probing Jamaica's Political Culture, volume 1: Main Trends in July – August 2006 Leadership and Governance Survey. Kingston, Jamaica: Centre for Leadership and Governance; 2007.
- [67] Bryman A, Cramer D. Quantitative data analysis with SPSS with 12 and 13: A guide for social scientists. London: Routledge; 2005.
- [68] Bodner TE. What Improves with Increased Missing Data Imputations? Structural Equation Modeling: A Multidisciplinary Journal, 2008; 15:4, 651-675, DOI: [10.1080/10705510802339072](https://doi.org/10.1080/10705510802339072).
- [69] Kang H. The prevention and handling of the missing data. Korean journal of anesthesiology, 2013; 64(5), 402–406. <https://doi.org/10.4097/kjae.2013.64.5.402>.
- [70] Graham JW. Missing data analysis: making it work in the real world. Annual Review of Psychology, 2009; 60:549–576.
- [71] Little RJ, D'Agostino R, Cohen ML, Dickersin K, Emerson SS, Farrar JT, Frangakis C, Hogan JW, Molenberghs G, Murphy SA, Neaton JD, Rotnitzky A, Scharfstein D, Shih W J, Siegel JP, Stern H. The prevention and treatment of missing data in clinical trials. The New England journal of medicine, 2012; 367(14), 1355–1360. <https://doi.org/10.1056/NEJMSr1203730>.
- [72] Berg B. Qualitative Research Methods for the social sciences. 4th ed. Allyn and Beacon; 2001.
- [73] Creswell WJ. Research design: Qualitative, quantitative and mixed methods approach 4th ed. University of Nebraska, Lincoln. SAGE Publication Inc; 2013.
- [74] Blalock HM, Jr. Conceptualization and measurement in the social sciences. London: SAGE Publication; 1982.
- [75] Crotty M. The foundations of social research: Meaning and perspective in the research process. London: SAGE; 2005.
- [76] Rubin HJ, Rubin IS. Qualitative Interviewing: The Art of Hearing Data. 2nd Edition, Sage Publications, London; 1995.

- [77] Leedy PD, Ormond JE. Practical Research Planning and Design. 9th Edition, Pearson Education International, Boston; 2010.
- [78] Burnham P. Research methods in politics, 2nd ed. London: Palgrave Macmillan; 2008.
- [79] Kvale S. Interview Views: An Introduction to Qualitative Research Interviewing. Thousand Oaks, CA: Sage Publications; 1996.
- [80] Cucinotta D, Vanelli M. WHO Declares COVID-19 a Pandemic. *Acta Biomed*. 2020 Mar 19;91(1):157-160. doi: 10.23750/abm.v91i1.9397. PMID: 32191675; PMCID: PMC7569573.
- [81] Centers for Disease Control and Prevention, National Center for Immunization and Respiratory Diseases (NCIRD). 2009 H1N1 Pandemic (H1N1pdm09 virus). Atlanta: CDC; 2019. <https://www.cdc.gov/flu/pandemic-resources/2009-h1n1-pandemic.html>
- [82] Centers for Disease Control and Prevention, National Center for Immunization and Respiratory Diseases (NCIRD). 1957-1958 Pandemic (H2N2 virus). Atlanta: CDC; 2019. <https://www.cdc.gov/flu/pandemic-resources/1957-1958-pandemic.html>
- [83] Centers for Disease Control and Prevention (CDC), National Center for Immunization and Respiratory Diseases (NCIRD). Past pandemics. Atlanta: CDC; 2018. <https://www.cdc.gov/flu/pandemic-resources/basics/past-pandemics.html>
- [84] Centers for Disease Control and Prevention, National Center for Immunization and Respiratory Diseases (NCIRD). 1968 Pandemic (H3N2 virus). Atlanta: CDC; 2019. <https://www.cdc.gov/flu/pandemic-resources/1968-pandemic.html>
- [85] Ministry of Health and Wellness (MoHW). COVID-19 Clinical Management Summary. Kingston: MoHW;20-2022. <https://www.moh.gov.jm/updates/coronavirus/covid-19-clinical-management-summary/>
- [86] Ministry of Health and Wellness (MoHW). COVID-19 at a glance: Using data prepared as at April 22, 2022. Kingston: MoHW;20-2022. https://www.moh.gov.jm/wp-content/uploads/2022/05/MOH_CovidataGlance.pdf.
- [87] World Health Organization (WHO). Global: Jamaica. Washington DC: WHO; 2023. <https://covid19.who.int/region/amro/country/jm>
- [88] Bucknor H. Man beaten after sneezing on bus. Kingston: Jamaica Star; 2020. <http://w.jamaica-star.com/article/news/20200319/man-beaten-after-sneezing-bus>
- [89]

APPENDIX I: CLIENT'S INSTRUMENT

Service delivery of Civil Society Organizations (CSOs) during the Coronavirus disease 2019 (COVID-19) pandemic Survey

Introduction

This study seeks to assess the service delivery of Civil Society Organizations (CSOs) during the Coronavirus disease 2019 (COVID-19) pandemic as well as to chart a path for the way forward in times of crisis. The Civil Society Organizations (CSOs) include the Jamaica AIDS Support for Life (JASL), Jamaicans for Justice (JFJ), Jamaican Network for Seropositives (JN+), Jamaica Community of Positive Women (JCW+), Transwave Jamaica, Equality For All Foundation, Eve for Life, Children First Agency, ASHE Company,

The researchers will protect your privacy and confidentially during the research process. Hence, this questionnaire does NOT require your personal information, and you may withdraw at your convenience if you feel the study violates any personal issues. Furthermore, there is no wrong or right answers to this survey and so we are soliciting your truth. This may be done by honestly responding to each question.

Instructions

You may feel at liberty to answer the questions correctly and truthfully because there are no wrong or right responses in this survey. To protect your privacy and confidentiality, no personal identifiers are placed on this survey. If you participate in this survey, it will be assumed that consent has been given by you.

1. Did you receive support from a CSO during the pandemic?

No [] Yes []

2. Where did you receive support? i. _____

ii. Not applicable _____

3. What kind of support did you receive? i. _____

ii. Not applicable _____

4. Rate the quality of the service(s) you received from the CSO **before** the COVID-19 pandemic

1-Very low, 2 – low, 3-moderate 4 – high, and 5– very high

5. Rate the quality of the service(s) you received from the CSO **during** the COVID-19 pandemic

1-Very low, 2 – low, 3-moderate 4 – high, and 5– very high

6. Was there a break in services you received from the CSO during the COVID-19 pandemic?

No [] Yes []

7. You are asked to rate the level of disruption of service delivery caused by each of the following COVID-19 mitigation measures.

1-Very low, 2 – Low, 3-Moderate 4 – High, and 5 – Very high

DETAILS	1	2	3	4	5
Curfew, restricted travel					
Mask wearing					
Reduced opening hours					
Social distancing					
Vaccination status					
Working from home					

8. Rate your general health status before the COVID-19 pandemic.

Excellent
Very good
Good
Moderate
Poor
Very poor

9. In the past six months, rate your general status.

Excellent
Very good
Good
Moderate

Poor
Very good

10. Did you experience any financial difficulties during the COVID-19 pandemic?

No [] Yes []

11. Did the COVID-19 pandemic influence your general health?

No [] Yes []

12. Did the service(s) received from the CSO during the COVID-19 pandemic help your general well-being

No [] Yes []

13. Were you pleased with the quality of service you received from the CSO during the COVID-19 pandemic?

No [] Yes []

14. Were you pleased with the quality of service received from the CSO before the COVID-19 pandemic?

No [] Yes []

15. Were there any other challenges, you experienced concerning the service received during the COVID-19 pandemic?

16. Give a per cent that indicates your level of satisfaction with the service you received during the COVID-19 pandemic _____

17. Give some recommendations that could be implemented to improve the service delivered by the CSO during a future pandemic.

For questions 18-22, draw a line under the answer you select.

18. Which age group do you belong to?

- . Under 18 years old
- . 18 – 27 years old
- . 28 – 37 years old
- . 38 – 47 years old
- . 48 – 57 years old
- . 58 – 67 years old
- . 68 years and older

19. What is your current union status?

- Single
- Common-law
- Married
- Separated (not legally divorced)
- Divorced (legally separated)
- Widowed

20. What is your highest level of education?

- None
- Primary/Preparatory
- High/Secondary
- College/University

21. What is your gender?

- Female
- Male

Non-binary

22. In which parish do you reside/live?

Kingston

St. Andrew

St. Thomas

Portland

St. Mary

St. Ann

Trelawny

St. James

Hanover

Westmoreland

St. Elizabeth

Manchester

Clarendon

St. Catherine

APPENDIX II: CSO'S INSTRUMENT

CSO Instrument: EXAMINING SERVICE DELIVERY SURVEY DURING THE COVID-19 PANDEMIC

Introduction

This study seeks to assess the service delivery of the Jamaicans for Justice (JFJ), Jamaican Network for Seropositives (JN+), Jamaica Community of Positives Women (JCW+), Transwave Jamaica, Equality for All Foundation, Eve for Life, ASHE company, Children Frist Agency, and Jamaica AIDS Support for Life (JASL) during the Coronavirus disease 2019 (COVID-19) pandemic as well as to chart a path for the way forward in times of crisis. The current study is primarily concerned about the previously mentioned issues from a truth perspective, and so the researchers will protect your privacy and confidentially during the research process.

Hence, the instrument does NOT require your personal information, and you may withdraw at your convenience if you feel the study violates any personal issues. Furthermore, this survey has no wrong or right answers, so we are soliciting your truth. This may be done by honestly responding to each question on the instrument.

Instructions

This survey comprises 26 items and we are seeking your participation in the data collection process. There are no correct answers, so you may complete the instrument based on your truth. To protect your privacy and confidentiality, no personal identifiers are placed on this survey. If you participate in this survey, it will be assumed that consent has been given by you. The anticipated time for completing this questionnaire is 10±5 minutes.

Ques 1 to 4: You are asked to rate the effectiveness of your services **before** the COVID-19 pandemic. Circle your selected rating.

1-Very low, 2 – low, 3-moderate 4 – high, and 5– very high

1. Collaboration, Communication and Capacity Building	1	2	3	4	5
2. Supporting infrastructure and sustainability of service delivery	1	2	3	4	5
3. Providing Social and material support	1	2	2	4	5
4. Access to Justice and Legal Redress	1	2	2	4	5

5. Was there a break in your services?

No [] Not really [] Yes []

6. Did the Coronavirus disease 2019 (COVID-19) pandemic hamper the extent of your services?

No [] Not really [] Yes []

7. If your response to Question 6 is yes, what aspect of your programme was most affected?

(a. Prevention []

(b. Treatment []

(c. Enabling Environment []

(d. Rights []

8. You are asked to rate the level of disruption of your service delivery caused by each of the following COVID-19 mitigation measures.

1-Very low, 2 – low, 3-moderate 4 – high, and 5 – very high

DETAILS	1	2	3	4	5
Curfew, restricted travel					
Mask wearing					
Reduced opening hours					
Social distancing					
Vaccination status					
Working from home					
Clients' geographic location					
Comorbidities					
Increased GBV					

9. Was the grant funding adequate to support your programme needs during the pandemic?

No [] Not really [] Yes []

10. You are asked to rate the level of disruption to your service delivery caused by each of the fallouts from the COVID-19 pandemic.

1-Very low, 2 – low, 3-moderate 4 – high, and 5– very high

DETAILS	1	2	3	4	5
Infected staff					
Absence of staff					
Mobility of staff					
Vaccination status					

Shortage of critical supplies caused by COVID-19					
Providing shelter,					
Increased community engagement;					
Supporting income generation;					
Provide back-to-school support for clients					
Procurement of Personal Protective Equipment					
Other					
Other					
Other					

11. You are asked to rate how did the grant from the Global Fund assist you in overcoming the disruption to your critical services?

1-Very low, 2 – low, 3-moderate 4 – high, and 5– very high

DETAILS	1	2	3	4	5
Operation of clinics					
Field visits					
Providing monetary support					
Providing food support					
Providing medication					
Other					
Other					
Other					

12. Did you employ any of the following approaches during the COVID-19 pandemic to carry out your services? (multi-responses acceptable)

Telehealth and telemedicine []
HIV self-testing []
Multi-month dispensing []
Community-based engagement in service implementation & monitoring []
Spacing of Visits []

13. Were other resources (i.e., financial, etc.) used to bolster services during the COVID-19 pandemic?

No [] Not really [] Yes []

QUESTIONS 14 TO 17 ARE RELATED TO PER CENT OF.

1=0%; 2=1-20%; 3=21%– 40%; 4=41%-60%; 5=61%-80%, and 6=81%-100%

14. What per cent of other budgets were used to bolster services during the COVID-19 pandemic?	1	2	3	4	5	6
15. What per cent of clients were used to serve during the COVID-19 pandemic?	1	2	3	4	5	6
16. What per cent of your clients have you served during the COVID-19 pandemic?	1	2	3	4	5	6
17. What per cent of the grant budget was spent?						

You are asked to rate the effectiveness of your services **during** the COVID-19 pandemic. Circle the rating selected.

1-Very low, 2 – low, 3-moderate 4 – high, and 5 – very high

18. Collaboration, Communication and Capacity Building	1	2	3	4	5	6
19. Supporting infrastructure and sustainability of service delivery	1	2	3	4	5	6
20. Providing Social and material support	1	2	3	4	5	6
21. Access to Justice and Legal Redress	1	2	3	4	5	6

22. State a per cent as it relates to the fulfilment of your services conducted during COVID-19 _____

23. State the extent to which you could have contributed more to your clients if the resources were available during the COVID-19 pandemic.

1-Very little, 2 – little, 3-moderate 4 – much, and 5 – a lot

24. What are the primary activities conducted by your organization during the pandemic through the grant? (specify): _____

25. Rate your engaged activities based on successful outcomes or completion.

1-Very low, 2 – low, 3-moderate 4 – high, and 5 – very high

26. List some challenges encountered during carrying out your activities.

APPENDIX III: GENERAL FOCUS GROUP FINDINGS

During the focus group session, there was a shared consensus among members of the CSOs that the government protocols (lockdown, no movement for non-essential workers), the lack of information and misinformation, initially brought the services to clients to a snail's pace and for some, it was a halt. The unpreparedness of the CSOs resulted in lowered service delivery, and this was noted by the clients. With time the CSOs implemented appropriate and suitable measures that allow for the provision of the necessary services.

In an interview with Dr Nicola Skyers, Director Disease Prevention and Control (Acting)/ Project Manager (HIV) at the Ministry of Health and Wellness (MoHW), she admitted that no one was prepared for the COVID-19 pandemic and concurred with the members of the CSOs that the fund was allocated during the mid-to-latter part of the pandemic; but that this should not have substantially hampered the service delivery of the organizations. From the interview with Dr Skyers and the focus group session with the members of the CSOs, there is a consensus that there would be some period of a break in service delivery; but the CSOs seem to have been handicapped for too long a period. The unexpectedness of the pandemic and not having all the scientific information at the beginning undoubtedly created a gulf between response and execution.

Dr Skyers opined, "There seemed to have been challenges in terms of the activities as per the approved work plan. In fact, some of the entities basically started implementing the activities in the second year which started in October 2021. So, lots of entities haven't implemented activities as per the year-one portion of the grant. I know that for some they had indicated that several of the activities were under budgeted". There seems to have been a glitch in communication between the MoHW and the CSOs in the execution of the work plan during the pandemic, and Dr Skyers postulated, "A consultant would have been recruited to work with the entities to develop their work plans, so these would then have been collated and submitted to the ministry for review to ensure that the activities match as well as we are within budget. So, this is a document that would have

been submitted by the CSOs for final approval. I'm not sure where there were some communication challenges, but some persons had in fact indicated that they weren't familiar with some of these activities.”

The slow response of the CSOs to the pandemic hampered not only the service delivery but intensified the fear of the clients, not knowing what was happening during the early stage of the pandemic. The lack of information on the virus, the ill health of some of the service delivers, and the low response of the CSOs interfered with the experiences of the clients during the pandemic. The clients were able to identify the gaps in service delivery and attributed this to the COVID-19 pandemic, suggesting that the pandemic significantly retarded service delivery.

Mr Cruickshank concurred with the CSOs and Dr Skyers that no entity was completely or partly prepared for the COVID-19 pandemic, and that people were responding as information developed. He chided the government for initially excluding the CSOs from the general discussion and planning responses to the pandemic and noted that still hampered the provision of services at the beginning of the pandemic. To support his perspective, Mr Cruickshank provided the example of the impulsive response of the government to the first reported and confirmed case of COVID-19. He articulated that the first confirmed case was in Bull Bay, St. Andrew, and the government order a shutdown of the community. This meant that some of the clients of the CSOs were unable to access services including medical supplies. On the onset, CSOs’ representatives were unable to visit clients within this community, and so this interfered with the outreach activities of the CSOs.

The interferences were sometimes outside of the scope of the CSOs to include the medication supplies and the medical distribution centres. Mr Cruickshank opined that medical supplies were restricted because the Global Supply Chain was unable to fulfil orders as obtained before the pandemic. Another aspect of interference was restriction based on the number of people

who could be served at clinics, hospitals, and DrugServ. Those challenges created a bottleneck at medical distribution sites as well as the early human mobility restrictions implemented by the government interfered with the service delivery to the clients. Mr Cruickshank noted that the number of unknowns that emerged due to the COVID-19 pandemic made it increasingly difficult for the CSOs to completely respond and address the myriad of challenges of the clients. Despite those issues, the CSOS were able to recalibrate their operations and new measures were implemented to address the realities of the clients.

Although the COVID-19 Response grant from the Global Fund (Fund) arrived in the midst of the COVID-19 pandemic, it played a major role in providing critical care packages, phone credit, financial support, medical and educational expenses, and other support for the clients of the CSOs in Jamaica. From the focus group session, before receiving the fund, money was reallocated from other programmes and some bureaucracies in the distribution process hampered a more extensive coverage of needs for the clients. Ms. McKenzie (pseudo name) opined that the *“bottlenecks caused by bureaucracy slowed the flow of funds to CSOs”* and this fact lowered responsiveness and full coverage of the CSOs. It seems like the *bottlenecks* to which the members of the CSOs speak were the late arrival of the funds and Dr Skyers concurred with this truth. However, she argued that they (CSOs) were instructed to divert funds to respond to the pandemic and that this would be reimbursed later, which was to be from the Global Fund

Despite the timing of the allocation of the Global Fund COVID-19 response grant, the members of the CSOs lauded the importance of the Fund to meeting the needs of their clients. The clients were equally and simultaneously concurring with the CSOs that the resources were allocated during the pandemic and that they were, for the most part, effective in meeting a large per cent of their needs. Dr Skyers opined:

They were given additional resources to build out their ICT infrastructure consumables and supplies and the phone cards, Internet packages etc. to support them in that kind of transition because face- to-face interactions were therefore limited particularly in year 1. So those things would have been taken into consideration. We knew that it definitely would have impacted, but I wouldn't say that is the major reason for any shortfall that they would have because they too would have transitioned to online services. In more recent times each organisation has been required to develop a risk register and that is now being integrated in the work that they do, and they do provide reports in regards to the risk register.

Dr Skyers' perspective provides some context for the utilization of social media, disbursement of sums for phone cards, and other methods during the pandemic to supply the needs of the clients. The views of the clients, and the benefactors, provide a comprehensive understanding of the reach/coverage and effective of engagement of the CSOs. Dr Skyers said *inter alia* "the indicators are designed to ensure people were receiving their test et cetera. But the indicators don't necessarily take into consideration the qualitative component of the work".

This study provides qualitative responses for the clients on how they were provided for during the pandemic. The clients indicated that they were provided for during the pandemic, except for a brief period of care.

On the other hand, Dr Skyers remarked, "It's very quantitative in terms of how the measurement is done --another person needed to get food packages and that sort of thing. But nobody really sits down and counts how many persons are able to stay on their medication because of the support they are getting. So, we do recognize the absence of the qualitative component. It's quite important, but based on how the global fund measures, like industries, it's purely numbers."

This study provides a quantitative aspect of the clients' views on benefits and operations during the pandemic, and majority of them indicated a high degree of satisfaction with the services they received during the period.

Despite the high level of satisfaction on service delivery in meeting the needs of the clients, Dr Skyers offered that there were stated objectives embodied in the Work Plans of the CSOs for assessing effectiveness of the COVID-19 response grant. The rationale for the identification of a Work Plan was for a quantitative assessment of the grant funding. So, it was important to develop a plan.

An elite interview with Mr Ivan Cruickshank of the Country Coordinating Mechanism in Jamaica (CCM) provided greater insight on some challenges encountered by the CSOs. He explained that although the work plan was developed by a consultant, it was in consultation with the CSOs. Mr Cruickshank noted that there were some issues with aspects of the work that would have accounted for some delays in execution. The executive director of JASL, Mrs. Kandasi Watson-Levermore, indicated that the Ministry of Health and Wellness was asked to provide a coordinator who would be responsible for the coordination of all activities relating to the COVID-19 response to clients, but this was never provided, or an allocation made in the budget for this expense. Mr Cruickshank explained that the non-provision of a coordinator hampered aspects of the work plan as JASL had to stretch the already meagre staff and assign someone to act as a coordinator. Clearly, this thwarted scheduled and timely execution of aspects of the work plan, which was somewhat outside of the CSOs' control.

The purpose of the work plan was to reach vulnerable populations during the pandemic. Dr Skyers aptly summarized the importance of the work plan, "There are no specific targets for the COVID-19 grant, we would have had targets under the work plan in terms of reaching the

various populations. So, there was nothing special. I mean it's an emergency. What we do routinely is that we have quarterly reports; and depending on the organization, we may have monthly meetings with them to review the implementation of the activities under the grant, not just limited to COVID-19, but also to the main grant that is being implemented. So, that is how we monitor the implementation of all our grants.”

Based on the quantitative aspect of this study, the reach of the response initiatives by the CSOs was extensive, and the coverage to meet the needs of the clients was over 70%. The coverage per cent indicates that there were underserved people, and this may be due to the increased expectations from the worsened socioeconomic state of the clients.

The COVID-19 pandemic was a surprise to people and entities in Jamaica. CSOs were occupied with a plethora of existing challenges prior to the pandemic, and this was exacerbated by the virus. Before COVID-19, the CSOs in Jamaica were actively engaged in addressing the concerns and needs of their clients, and this was done with limited existing resources. During the focus group session, all the CSOs reported that they were not prepared for the COVID-19 pandemic and that they had to divert resources from other programmes/projects in order to immediately respond to the pandemic. Dr Skyers from the Ministry of Health and Wellness (MoHW) echoed the sentiments of the members of the CSOs that no one was prepared for the pandemic.

The CSOs in attendance indicated that they had a strategic disaster protocol; but it did not include a pandemic. They postulated that the COVID-19 was different from the issues contained in existing disaster protocols and they had to improvise to deal with the new realities. One CSO indicated that he experienced severe challenges to access information on the pandemic and initially

he was excluded from policy planning meetings with the government and/or the Ministry of Health and Wellness. The perspective of Amos (pseudo name) was also echoed by all the agents present at the session. During the early stage of the pandemic, the lack of information and misinformation on the virus coupled with the psychology of fear for the virus as well as governmental restrictions made it different from challenges faced in the past. All those issues caused a break in service, and they intensified the expectations of the clients.

Dr Skyers opined: “To provide resources to help CSOs to mitigate the challenges caused by the pandemic in delivering benefits to beneficiaries and that embedded in the purpose of the Global Fund was meeting the needs of the served clients. The fact is the expectations of the clients and the reality of their situation demanded more resources, which is somewhat understandable according to Dr. Skyers. The reality is the CSOs had desired a system of ensuring that all the clients are served and that they would not roam all the agencies for the same benefits.

The Global Fund’s assistance came towards the end of the heightened period of the pandemic. The much-needed fund replaced the other diverted funds. Many of the employees of the CSOs were ill, and there were no funds to acquire other skilled workers. The issue of human resources was crippled during the pandemic, this was unplanned and so innovative measures had to be employed to supply the needs of the clients. The CSOs reiterated that not having reserved funds, an established pandemic protocol, and money for administrative expenses crippled the initial response of the agencies. Phone credits and social media platforms were employed to engage the clients, and this meant managing a new reality. The CSOs opined that many lessons have been learnt from the pandemic; but that there is an urgent need for a robust pandemic protocol that includes them as critical elements in a first response protocol.